

ENGLISH FOR ONCOLOGISTS

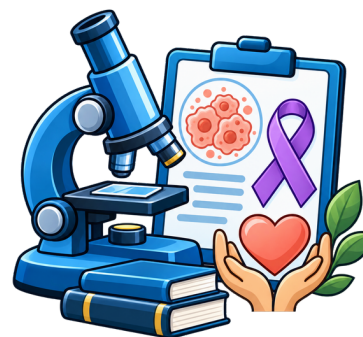
Oncology English

Explain precisely. Leave room to respond.

Phil Stilwell

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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For oncologists, oncology fellows, and physicians coordinating cancer care.

2026 edition

Eight scenarios. Eight skills.

Eight oncology encounters practice difficult news, staging language, biomarker uncertainty, treatment choices, symptom escalation, response assessment, trial discussions, and care goals.

01 Explaining a new cancer diagnosis

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02 Separating stage, grade, and prognosis

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An option to discuss, not an obligation | Explain research participation while separating eligibility, consent, and proven benefit.

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Precision without emotional distance.

Oncology conversations often carry unfamiliar terminology and strong emotion at the same time. These original cases practice small information steps, explicit uncertainty, and a clear next conversation.

Ask how much information is wanted

Check the patient's understanding and information preference. Pause after difficult news instead of covering silence with more terminology.

"Would you like the main finding first, then time for questions?"

Separate related technical terms

Stage, grade, response, remission, and cure answer different questions. State what the available evidence supports and what remains unresolved.

"The pathology establishes the diagnosis; staging is still incomplete."

Make the treatment aim explicit

A discussion of benefit needs a stated goal and uncertainty. Avoid describing control, symptom relief, and cure as interchangeable outcomes.

"The aim of this option is disease control; it is not a guarantee."

Connect support with active care

Pain, distress, practical needs, and family communication belong in cancer care. A supportive-care referral does not by itself mean anticancer treatment is ending.

"We can address symptoms alongside the cancer treatment discussion."

PROFESSIONAL BOUNDARIES

All people, records, figures, and encounters are fictional. This is professional English practice, not medical advice, clinical training, a treatment protocol, or certification. Use current evidence, authorized supervision where required, local law, and your actual clinical and emergency procedures. Do not use an exercise to decide that symptoms can safely wait, select treatment, or calculate a dose. Communication about privacy, consent, and professional duties depends on jurisdiction and setting. Use qualified language support and protect real patient information.

The first clear sentence

A Read the situation

Deliver an established finding in small steps while responding to emotion and information preferences.

Dr Imani meets Maya after a pathology report confirms a malignant tumor. Staging is incomplete, and no treatment recommendation or prognosis estimate has been established. Maya wants the main finding first and then a pause. Maya has not yet decided whether a relative should join. The clinician must give the finding plainly, acknowledge the response, and separate the diagnosis from unanswered questions.

PEOPLE / Dr Imani: Oncologist | Maya: Patient

A PAUSE IS PART OF THE CONVERSATION

Silence after difficult news need not be filled with statistics. Check what the person wants next and who they want involved, without assuming either preference.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is established?

- | | |
|------------------------------|---|
| A A completed stage | B A guaranteed treatment outcome |
| C A survival estimate | D A malignant tumor on pathology |

2. What does Maya request?

- | | |
|--|--------------------------------------|
| A The main finding followed by a pause | B Every statistic immediately |
| C Disclosure to all relatives automatically | D No explanation ever |

3. What family decision is unfinished?

- | | |
|---|--|
| A Whether the relative is the decision-maker | B Whether privacy no longer applies |
| C Whether a relative should join | D Whether all relatives consented |

[Answers and explanations: page 85.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

pathology report

Laboratory interpretation of examined tissue or cells.

IN USE review the pathology report

biopsy

Removal of tissue or cells for examination.

IN USE explain the biopsy finding

malignancy

Cancerous disease characterized by malignant cells or tissue.

IN USE explain a confirmed malignancy

histology

Microscopic study of tissue structure and features.

IN USE review the histology

primary tumor

Tumor at the site where the cancer began.

IN USE identify the primary tumor

diagnostic disclosure

Communication of an established diagnosis to the patient.

IN USE prepare for diagnostic disclosure

warning statement

Brief introduction preparing a listener for difficult information.

IN USE use a clear warning statement

information preference

How much detail a person wants and how they want it presented.

IN USE check the information preference

emotional acknowledgment

Recognition of the listener's expressed feelings or response.

IN USE offer emotional acknowledgment

prognostic uncertainty

Incomplete ability to predict an individual's future disease course.

IN USE explain prognostic uncertainty

staging workup

Investigations used to establish the extent of cancer.

IN USE complete the staging workup

multidisciplinary team

Professionals from different specialties contributing to a care discussion.

IN USE consult the multidisciplinary team

Precision check

The diagnosis is established in this case, but stage, treatment, and prognosis are not. Do not soften a confirmed finding into vagueness or turn it into an unsupported prediction.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

second opinion

Assessment or advice from another qualified clinician.

IN USE arrange a second opinion

care navigator

Professional helping a person move through services and appointments.

IN USE contact the care navigator

support person

Individual the patient wants involved for assistance.

IN USE invite a chosen support person

diagnostic certainty

Degree to which the diagnosis is established by evidence.

IN USE distinguish diagnostic certainty

information overload

More information than a listener can usefully process at one time.

IN USE avoid information overload

follow-up consultation

Later meeting to review findings, options, or unresolved questions.

IN USE arrange a follow-up consultation

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Ask permission before involving a relative. A support person is not automatically a substitute decision-maker.

Phrases you can actually use

C Notice the language / 1 of 2

PREPARE FOR THE FINDING

"I am afraid the result is not what we hoped for."

ALLOW A PAUSE

"We can stop here for a moment."

SEPARATE QUESTIONS

"We know the diagnosis; we do not yet know the full extent."

LIMIT A PREDICTION

"I cannot give an individual outlook from this finding alone."

NAME THE DIAGNOSIS

"The tissue examination confirms a malignant tumor."

ACKNOWLEDGE EMOTION

"I can see that this is a great deal to take in."

ASK ABOUT SUPPORT

"Would you like someone you choose to join us?"

OFFER CONTINUITY

"We will arrange another conversation when the remaining results are available."

Confirms

Appropriate here because the fictional pathology finding is established, unlike the incomplete stage.

Not yet

Keeps an unanswered question visible without treating uncertainty as a negative result.

Would you like

Invites a preference rather than assuming how much information or family involvement is wanted.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

MAKE SPACE

"We can pause before moving to the next point."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

EXPLAIN A TERM

"In everyday language, that means the following."

CHECK YOUR EXPLANATION

"Let me explain that another way."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening is clearest?

- A** Everything is definitely curable.
- B** The stage is already complete.
- C** The tissue examination confirms a malignant tumor.
- D** Something might be a little unusual, but who knows.

2. Which response respects Maya's preference?

- A** Invite relatives without asking.
- B** Demand an immediate treatment decision.
- C** Pause after the main finding.
- D** Continue through every statistic immediately.

3. Which conclusion exceeds the facts?

- A** A later discussion is needed.
- B** The diagnosis alone establishes Maya's individual prognosis.
- C** Staging remains incomplete.
- D** The pathology confirms malignancy.

4. What should precede family involvement?

- A** Assuming every relative may receive details
- B** Treating attendance as legal authority
- C** Posting the result publicly
- D** Clarifying Maya's wishes and the appropriate permissions

[Answers and explanations: page 85.](#)

The first clear sentence

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

information overload / staging workup / malignant tumor / pathology report / support person / prognostic uncertainty / biopsy / second opinion / care navigator / information preference

- 01 **Dr Imani:** Before we discuss the report, how much detail would you like in this first conversation?
- 02 **Maya:** The main finding first, please. My ¹ _____ may change once I have heard it.
- 03 **Dr Imani:** I am afraid the result is not what we hoped for. The tissue examination confirms a ² _____.
- 04 **Maya:** Please give me a moment. I knew the ³ _____ might find something, but hearing that is different.
- 05 **Dr Imani:** Of course. We can pause. I can see this is a great deal to take in.
- 06 **Maya:** When I am ready, could you explain what the ⁴ _____ tells you and what it does not?
- 07 **Dr Imani:** It establishes the cancer diagnosis. It does not yet give us every answer about the extent or treatment.
- 08 **Maya:** So the ⁵ _____ is a separate part of understanding what happens next?
- 09 **Dr Imani:** Yes. We need those remaining details before completing the treatment discussion or estimating your individual outlook.
- 10 **Maya:** I was about to ask how long I have. I hear that there is still ⁶ _____.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Dr Imani:** I will answer as honestly as I can, including what we do not know. I will not invent a number.
- 12 **Maya:** Thank you. I might want my brother here, but I have not chosen a ⁷ _____ for this conversation yet.
- 13 **Dr Imani:** You can decide. We will discuss what you want shared rather than assume that every family member is involved.
- 14 **Maya:** Could someone help me understand the appointments? I am finding even ordinary arrangements difficult to absorb.
- 15 **Dr Imani:** Our ⁸ _____ can help with those arrangements while we keep the clinical questions with the appropriate team.
- 16 **Maya:** I do not want a large pack of unexplained information to be my only connection with the team.
- 17 **Dr Imani:** That would risk ⁹ _____. We will give manageable information and a clear contact route.
- 18 **Maya:** I may also want another clinician to review the findings once I understand the next steps.
- 19 **Dr Imani:** We can discuss a ¹⁰ _____ and arrange a follow-up conversation. You do not need every decision settled in this first exchange.
- 20 **Maya:** For now, I understand the diagnosis is confirmed and the full extent is not. Please let us stop there briefly.

ANSWERS / EXPLANATIONS: PAGE 86

1. information preference | 2. malignant tumor | 3. biopsy | 4. pathology report | 5. staging workup | 6. prognostic uncertainty | 7. support person | 8. care navigator | 9. information overload | 10. second opinion

Check who should join

F Say it

Alex wants partner Ren to attend the next results conversation but has not authorized disclosure to other relatives. The next meeting will review outstanding findings; no outcome is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

Ren / supplied / outstanding / relatives

Alex: "I would like ¹ _____ to join."

Clinician: "The permission does not automatically include other
² _____."

Alex: "Some findings remain ³ _____."

Clinician: "No outcome has been ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. Ren | 2. relatives | 3. outstanding | 4. supplied

Rehearse with a fixed script

1. Read the corrected disclosure slowly. Pause after the finding and after Maya's request for time.
2. Swap roles. Repeat the distinction between a confirmed diagnosis and an incomplete stage without adding a prognosis.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Two terms on one report

A Read the situation

Explain closely related oncology terms without converting one into another.

Theo has a pathology report that includes a tumor grade. Imaging needed for staging is still pending. Dr Malik must explain that grade concerns microscopic features while stage describes extent using the applicable cancer-specific system. No grade value, stage, or individual survival estimate is supplied. Theo has mistaken the grade for a completed stage.

PEOPLE / Theo: Patient | Dr Malik: Oncologist

CORRECT THE INTERPRETATION, NOT THE PERSON

Technical terms can appear together on reports while answering different questions. Use a clear contrast and a brief check-back rather than telling the patient they read it incorrectly.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which item is available?

- | | |
|---|--|
| A A completed staging conclusion | B A supplied survival estimate |
| C All pending imaging | D A pathology report containing grade |

2. What is pending?

- | | |
|--------------------------------|---------------------------------------|
| A Whether Theo attended | B The word grade on the report |
| C A confirmed cure | D Imaging needed for staging |

3. What has Theo confused?

- | | |
|---|---|
| A Two confirmed diagnoses | B Grade with stage |
| C A treatment cycle with a payment | D A medicine with an appointment |

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

tumor grade

Classification of microscopic tumor features under the relevant grading system.

IN USE explain tumor grade

cancer stage

Description of cancer extent under the applicable staging system.

IN USE establish the cancer stage

differentiation

Degree to which tumor cells resemble normal cells of their origin.

IN USE describe cellular differentiation

local extent

Spread of a tumor within its original area.

IN USE assess local extent

regional lymph node

Lymph node in the region draining or associated with the tumor site.

IN USE assess regional lymph nodes

distant metastasis

Cancer spread to a site distant from the primary tumor.

IN USE evaluate distant metastasis

clinical stage

Stage based on clinical evaluation before definitive pathological staging when applicable.

IN USE record the clinical stage

pathological stage

Stage incorporating surgical and pathological findings where applicable.

IN USE distinguish pathological stage

TNM classification

Cancer-specific classification describing tumor, regional nodes, and distant metastasis.

IN USE explain the TNM classification

stage group

Overall stage category combining applicable staging information.

IN USE confirm the stage group

staging investigation

Test used to help determine cancer extent.

IN USE arrange a staging investigation

imaging finding

Observation reported from a diagnostic image.

IN USE review an imaging finding

Precision check

Grade and stage are not synonyms. Staging systems differ by cancer type; do not assign a TNM category or stage from a vocabulary definition.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

indeterminate finding

Finding whose nature or significance is not yet established.

IN USE explain an indeterminate finding

nodal involvement

Presence of cancer in lymph nodes.

IN USE assess nodal involvement

localized disease

Cancer confined to a defined local site in the relevant context.

IN USE describe localized disease

metastatic disease

Cancer that has spread to distant body sites.

IN USE discuss metastatic disease

prognostic factor

Feature associated with a disease's expected course or outcome.

IN USE interpret a prognostic factor

staging completion

Point at which the necessary staging information has been assembled.

IN USE confirm staging completion

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

An indeterminate imaging finding is not automatically metastatic disease. Preserve the report's uncertainty and the need for clinical interpretation.

Phrases you can actually use

C Notice the language / 1 of 2

INTRODUCE THE CONTRAST

"These two terms answer different questions."

EXPLAIN STAGE

"Stage describes the extent using the system for this cancer."

AVOID A SHORTCUT

"We cannot convert the grade into a stage."

LIMIT PROGNOSIS

"A stage label is not an exact prediction for one person."

EXPLAIN GRADE

"Grade describes microscopic features of the tumor."

NAME THE MISSING EVIDENCE

"The required imaging is still pending."

PRESERVE UNCERTAINTY

"Indeterminate does not mean confirmed spread."

CHECK THE CONTRAST

"Which term describes extent, and which describes the tissue features?"

Describes versus predicts

A classification describes evidence; it does not predict an individual's exact outcome.

Still pending

States that an investigation is unfinished rather than negative.

Not automatically

Blocks an invalid inference while leaving room for appropriate assessment.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which contrast is correct?

- A** Stage always means cure.
- B** Grade alone proves distant spread.
- C** Grade concerns microscopic features; stage concerns extent.
- D** Grade and stage are interchangeable.

2. What does pending imaging mean?

- A** The required result is not yet available.
- B** Spread has been excluded.
- C** Spread has been confirmed.
- D** Imaging has been canceled permanently.

3. Which wording preserves an indeterminate finding?

- A** It must be deleted.
- B** Its significance is not yet established.
- C** It is definitely metastatic.
- D** It is definitely harmless.

4. Which claim is unsupported?

- A** The terms need explaining.
- B** The grade supplies Theo's exact survival time.
- C** The report contains grade.
- D** Staging remains incomplete.

Answers and explanations: page 87.

Two terms on one report

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Tumor grade / regional lymph node / Distant metastasis / prognostic factor / local extent /
Differentiation / staging completion / indeterminate finding / TNM classification / cancer stage

- 01 **Theo:** My report gives a grade. I thought that meant you already knew the stage of the cancer.
- 02 **Dr Malik:** ¹ _____ describes microscopic features. Stage answers a different question about the extent of disease.
- 03 **Theo:** Then I may have combined two separate pieces of information when I explained the report at home.
- 04 **Dr Malik:** That is understandable. The ² _____ is not complete while we are waiting for the required imaging.
- 05 **Theo:** I also saw a word about how much the cells resemble ordinary tissue. Where does that fit?
- 06 **Dr Malik:** ³ _____ concerns that resemblance. It can inform grading, but it does not by itself tell us where disease is present.
- 07 **Theo:** What does the staging investigation need to establish beyond what the microscope has shown?
- 08 **Dr Malik:** Depending on the cancer, it examines features such as ⁴ _____, nearby nodes, and possible disease elsewhere.
- 09 **Theo:** So a nearby lymph node and a finding in a distant organ would not be described identically?
- 10 **Dr Malik:** Correct. A ⁵ _____ and a distant site have different roles in the applicable staging system.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Theo:** I have heard people use the word metastatic for any unusual spot. Is that too definite?
- 12 **Dr Malik:** Yes. An ⁶ _____ has not yet been characterized. We must not relabel uncertainty as confirmed spread.
- 13 **Theo:** That distinction helps. I would rather the report's uncertainty stay visible than be translated into the worst possibility.
- 14 **Dr Malik:** Exactly. ⁷ _____ is a specific conclusion, not a synonym for every finding that needs further review.
- 15 **Theo:** Will every cancer use the same letters and numbers once those investigations are complete?
- 16 **Dr Malik:** No. ⁸ _____ applies in cancer-specific ways, and some cancers use other systems.
- 17 **Theo:** And even when the stage is known, it will not tell you exactly what happens to me personally?
- 18 **Dr Malik:** Correct. A ⁹ _____ informs a discussion; it is not an exact individual timetable.
- 19 **Theo:** Then I can say the grade is reported, but the stage still needs the remaining investigation results.
- 20 **Dr Malik:** Yes. We will confirm ¹⁰ _____ and explain the conclusion when the evidence is available.

ANSWERS / EXPLANATIONS: PAGE 88

1. Tumor grade | 2. cancer stage | 3. Differentiation | 4. local extent | 5. regional lymph node | 6. indeterminate finding | 7. Distant metastasis | 8. TNM classification | 9. prognostic factor | 10. staging completion

Keep an uncertain scan finding uncertain

F Say it

A report calls a lung finding indeterminate. The oncology team is reviewing it. No metastatic diagnosis or stage change has been established.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

confirmed / established / indeterminate / reviewing

Patient: "The report calls the finding ¹ _____."

Clinician: "The team is still ² _____ it."

Patient: "Spread has not been ³ _____."

Clinician: "No stage change is ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. indeterminate | 2. reviewing | 3. established | 4. confirmed

Rehearse with a fixed script

1. Read the corrected terminology discussion. Contrast grade, stage, and an indeterminate finding.
2. Swap roles. Repeat the final summary without supplying a stage or prognosis that the case does not contain.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A possible match is not a promise

A Read the situation

03

Explain the difference between a molecular finding, a relevant option, and treatment eligibility.

A tumor biomarker report identifies a potentially relevant molecular alteration for Quinn. Dr Sato has not established that a particular treatment is appropriate or available. Clinical context, the strength of evidence, and access need review. The report is tumor testing; no inherited finding has been established. Quinn hopes the result guarantees an effective targeted treatment.

PEOPLE / Dr Sato: Oncologist | Quinn: Patient

HOPE DOES NOT REQUIRE A GUARANTEE

Acknowledge why a possible treatment match matters. Explain the remaining checks without presenting them as bureaucratic obstacles or erasing the potential relevance.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the report identify?

- A Guaranteed treatment benefit
- C An approved appointment date

- B Confirmed inherited risk
- D A potentially relevant tumor alteration

2. What is not established?

- A Suitability or availability of a particular treatment
- C That Quinn has questions

- B That a report exists
- D That clinical review is needed

3. What kind of testing is described?

- A Tumor testing
- C Testing every relative

- B Confirmed germline testing
- D A completed trial enrollment

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

03

Read the definitions and say the common word combinations aloud.

biomarker

Measurable feature providing information about a biological process or disease.

IN USE interpret a tumor biomarker

molecular alteration

Change in a molecular feature such as DNA sequence or gene activity.

IN USE characterize a molecular alteration

targeted therapy

Treatment directed at a relevant molecular target involved in disease.

IN USE evaluate a targeted therapy

somatic variant

Genetic change acquired in body cells rather than inherited through germ cells.

IN USE explain a somatic variant

germline variant

Genetic change in the inherited genetic material that may be passed to offspring.

IN USE assess a germline variant

variant of uncertain significance

Genetic change whose clinical meaning is not established.

IN USE report a variant of uncertain significance

actionability

Degree to which a finding can usefully inform a clinical action.

IN USE assess clinical actionability

evidence level

Strength or category of support for a clinical claim or action.

IN USE review the evidence level

companion diagnostic

Test providing information essential for appropriate use of a corresponding treatment.

IN USE verify a companion diagnostic

tumor profiling

Characterization of molecular features of a tumor.

IN USE review tumor profiling

sequencing panel

Test examining a selected set of genetic targets.

IN USE interpret a sequencing panel

specimen adequacy

Whether a sample meets the requirements for a reliable analysis.

IN USE confirm specimen adequacy

Precision check

A molecular alteration does not guarantee a useful treatment match or response. A tumor finding is not automatically an inherited finding.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

tumor heterogeneity

Differences among cells or regions within or between tumors.

IN USE discuss tumor heterogeneity

acquired resistance

Reduced treatment sensitivity developing during or after exposure.

IN USE investigate acquired resistance

predictive biomarker

Feature associated with likelihood of response to a particular treatment.

IN USE interpret a predictive biomarker

genetic counseling

Professional support in understanding genetic findings and their implications.

IN USE arrange genetic counseling

testing limitation

Restriction on what a test can detect or establish.

IN USE explain a testing limitation

treatment access

Practical ability to obtain an appropriate treatment.

IN USE clarify treatment access

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Uncertain significance means the clinical meaning remains unresolved. Do not call a variant actionable solely because it appears in a detailed report.

Phrases you can actually use

C Notice the language / 1 of 2

03

ACKNOWLEDGE HOPE

"I understand why a possible match feels important."

SEPARATE TWO STEPS

"Finding a target and choosing a treatment are different steps."

EXPLAIN REVIEW

"We need to assess the evidence and your clinical circumstances."

NAME ACCESS

"We also need to establish whether an appropriate option is available."

STATE THE FINDING

"The report identifies a tumor alteration that may be relevant."

LIMIT A PROMISE

"This does not guarantee that a treatment will work."

SEPARATE INHERITANCE

"Tumor testing does not automatically establish an inherited change."

PROTECT UNCERTAINTY

"Uncertain significance is not the same as proven actionability."

Potentially relevant

States possible significance without claiming a confirmed treatment indication.

Does not automatically

Prevents a tumor result from being treated as proof of inherited risk.

Need to establish

Marks a necessary unresolved check rather than a promised outcome.

Phrases you can actually use

C Notice the language / 2 of 2

03

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement preserves the result accurately?

- A** The alteration may be relevant; suitability still needs review.
- B** A targeted treatment is guaranteed to work.
- C** Every reported variant is actionable.
- D** Testing proves every relative is affected.

2. Which distinction matters for relatives?

- A** Tumor findings are not automatically inherited findings.
- B** Every somatic variant is inherited.
- C** All tumor tests test the entire family.
- D** Germline and somatic mean identical things.

3. What does uncertain significance mean?

- A** The report is always wrong.
- B** The condition is certainly inherited.
- C** Clinical meaning is not established.
- D** A treatment must be started immediately.

4. Which additional issue belongs in the discussion?

- A** Automatic trial enrollment
- B** Evidence, clinical context, and treatment access
- C** Only the color of the report
- D** A guaranteed response date

Answers and explanations: [page 89](#).

A possible match is not a promise

E Conversations / turns 1-10

03

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

genetic counseling / evidence level / germline variant / targeted therapy / testing limitation / somatic variant / variant of uncertain significance / treatment access / molecular alteration / actionability

- 01 **Dr Sato:** The tumor report is available. What have you understood from it so far, and what would you like clarified?
- 02 **Quinn:** It lists a ¹ _____. I hoped that meant there was a medicine certain to work.
- 03 **Dr Sato:** I understand that hope. The finding may be relevant, but it is the start of another clinical discussion.
- 04 **Quinn:** Is this the difference between a finding and its ² _____, rather than just a complicated way to say the same thing?
- 05 **Dr Sato:** Yes. We need to review the supporting evidence and whether the finding fits an appropriate option for you.
- 06 **Quinn:** I would like you to explain the ³ _____. A long report can look more certain than the information really is.
- 07 **Dr Sato:** That is a fair request. A possible target does not guarantee a response to the associated treatment.
- 08 **Quinn:** Then ⁴ _____ describes how an option works, not a promise of benefit for me.
- 09 **Dr Sato:** Correct. The clinical setting and other factors matter, and we also need to establish availability.
- 10 **Quinn:** Please include ⁵ _____ in that discussion. I cannot assume something listed in a report is obtainable here.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 28.

- 11 **Dr Sato:** We will. You also asked whether this result means your children have the same genetic change.
- 12 **Quinn:** Yes. I was unsure whether a ⁶ _____ was the same as something inherited through the family.
- 13 **Dr Sato:** It is not the same category. This is tumor testing; it does not automatically establish an inherited finding.
- 14 **Quinn:** So a ⁷ _____ would require the appropriate separate interpretation and, where indicated, further assessment.
- 15 **Dr Sato:** Exactly. We can discuss a genetics referral if appropriate, without declaring an inherited result that we do not have.
- 16 **Quinn:** I also noticed a different entry marked ⁸ _____. I should not treat that as another proven treatment target.
- 17 **Dr Sato:** Correct. We will explain the limits of the test as well as the findings it reports.
- 18 **Quinn:** A ⁹ _____ is important to me because I do not want a negative entry interpreted as an answer to every question.
- 19 **Dr Sato:** We will review those limits, the possible options, and the next steps together before making recommendations.
- 20 **Quinn:** And ¹⁰ _____ can help if the inherited-risk question needs specialist discussion, rather than me guessing from the report.

ANSWERS / EXPLANATIONS: PAGE 90

1. molecular alteration | 2. actionability | 3. evidence level | 4. targeted therapy | 5. treatment access | 6. somatic variant | 7. germline variant | 8. variant of uncertain significance | 9. testing limitation | 10. genetic counseling

A result with insufficient sample

F Say it

A profiling report says the sample was insufficient for one analysis. The result for that target is unavailable, not negative. A clinician will discuss whether further testing is appropriate.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

review / insufficient / negative / unavailable

Patient: "The sample was ¹ _____ for that analysis."

Clinician: "The target result is ² _____."

Patient: "That is not the same as a ³ _____ result."

Clinician: "Further testing needs clinical ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. insufficient | 2. unavailable | 3. negative | 4. review

Rehearse with a fixed script

1. Read the corrected biomarker discussion. Contrast a finding, a possible option, and a guaranteed response.
2. Swap roles. Repeat the somatic and germline distinction without declaring an inherited result.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

What is this option trying to achieve?

A Read the situation

Explain treatment goals and tradeoffs without promising a response or presenting consent as a formality.

Dr Bell discusses a proposed treatment option with Jordan. In this fictional case its stated aim is disease control, not a promised cure. The benefits, burdens, alternatives, and practical effects still need an individual discussion. Jordan prioritizes being able to care for a partner. No regimen, dose, response probability, or final decision is supplied.

PEOPLE / Jordan: Patient | Dr Bell: Oncologist

ASK WHAT THE GOAL MEANS IN DAILY LIFE

A treatment goal has both a clinical meaning and a personal significance. A patient may value time at home, symptom relief, or a particular activity differently from the clinician's initial assumption.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the proposed aim?

- A A completed response
- C Disease control

- B A fixed survival extension
- D A guaranteed cure

2. What matters particularly to Jordan?

- A Avoiding every future discussion
- C A decision already made

- B Caring for a partner
- D A specific dose supplied here

3. What remains undecided?

- A The final treatment choice
- C Whether a discussion is taking place

- B Whether Jordan has a partner
- D Whether the option has an aim

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

treatment intent

Goal a treatment is intended to pursue.

IN USE clarify treatment intent

curative intent

Treatment aim of eliminating the cancer.

IN USE explain curative intent

disease control

Limiting cancer growth or spread as a treatment goal.

IN USE discuss disease control

neoadjuvant treatment

Treatment given before the main local treatment in an applicable plan.

IN USE explain neoadjuvant treatment

adjuvant treatment

Additional treatment after a primary treatment to reduce recurrence risk when appropriate.

IN USE discuss adjuvant treatment

systemic therapy

Treatment acting throughout the body rather than only at one local site.

IN USE explain systemic therapy

local treatment

Treatment directed at a particular body area.

IN USE discuss local treatment

treatment regimen

Planned combination and schedule of treatment components.

IN USE review the treatment regimen

treatment cycle

Repeated unit of a planned treatment schedule.

IN USE explain a treatment cycle

performance status

Clinical description of a person's activity and functional ability.

IN USE assess performance status

quality of life

Person's experienced well-being across relevant areas of life.

IN USE discuss quality of life

treatment burden

Physical, practical, and emotional demands associated with treatment.

IN USE assess treatment burden

Precision check

Curative intent states an aim, not a guarantee. Disease control and symptom relief are also meaningful but different goals.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

benefit-risk balance

Weighing expected benefits against possible harms.

IN USE explain the benefit-risk balance

alternative option

Another reasonable approach available for discussion.

IN USE compare alternative options

supportive treatment

Intervention addressing symptoms or treatment-related difficulties.

IN USE coordinate supportive treatment

informed consent

Voluntary agreement after an appropriate explanation and opportunity for questions.

IN USE support informed consent

decision capacity

Ability to make the relevant decision under the applicable assessment.

IN USE assess decision capacity

caregiving responsibility

Duties involved in helping another person with care.

IN USE discuss caregiving responsibilities

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A signature does not replace a discussion of the proposed intervention, alternatives, benefits, risks, and the person's questions. This exercise supplies no treatment selection rule.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE AIM

"The aim of this option is to control the disease."

ASK ABOUT DAILY LIFE

"How would the treatment demands affect caring for your partner?"

INVITE ALTERNATIVES

"Let us compare the reasonable alternatives for your circumstances."

CHECK THE GOAL

"What do you understand this option is trying to achieve?"

AVOID A GUARANTEE

"An intended benefit is not a promised outcome."

EXPLAIN THE BURDEN

"We should discuss visits, symptoms, and practical demands together."

KEEP CHOICE OPEN

"You have not made a final decision in this discussion."

RESPECT QUESTIONS

"Questions are part of the consent discussion, not an obstacle to it."

The aim is

States intended purpose without claiming an achieved result.

Would affect

Explores a possible practical consequence before a final decision.

Not a guarantee

Prevents treatment-intent language from becoming a promised outcome.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement accurately names the goal?

- A** This option guarantees cure.
- B** A response has already occurred.
- C** The outcome is certain.
- D** This option aims at disease control.

2. Which question elicits a relevant priority?

- A** How would the treatment demands affect caring for your partner?
- B** Why does your partner matter?
- C** Can we ignore your home responsibilities?
- D** You agree that practical issues are irrelevant?

3. Which account of consent is incomplete?

- A** Questions need attention.
- B** Alternatives need discussion.
- C** The decision must be voluntary.
- D** A signature alone proves understanding.

4. Which distinction is correct?

- A** Curative intent is an aim, not a guarantee.
- B** Disease control means certain cure.
- C** Treatment burden means only cost.
- D** Quality of life means only a scan result.

Answers and explanations: [page 91](#).

What is this option trying to achieve?

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

benefit-risk balance / treatment intent / caregiving responsibility / Disease control / treatment cycle / Treatment burden / alternative option / quality of life / Informed consent / treatment regimen

- 01 **Jordan:** Before we go through appointments, I need to understand what you hope this treatment will do.
- 02 **Dr Bell:** The ¹ _____ here is disease control. I am not describing it as a guaranteed cure.
- 03 **Jordan:** That is clearer than saying it will help. Does control mean stopping everything permanently?
- 04 **Dr Bell:** ² _____ means trying to limit growth or spread; the possible benefit and uncertainty need an individual discussion.
- 05 **Jordan:** I care for my partner. Repeated hospital visits could affect both of us, not only me.
- 06 **Dr Bell:** Your ³ _____ belongs in this conversation. Let us examine those demands alongside the clinical aims.
- 07 **Jordan:** I do not want practical difficulties to sound as though I am refusing treatment without listening.
- 08 **Dr Bell:** They do not. ⁴ _____ includes visits and disruption as well as physical effects.
- 09 **Jordan:** Could you explain the proposed schedule, then distinguish what is definite from what may change?
- 10 **Dr Bell:** We would review the ⁵ _____ and the circumstances in which it might need review.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Jordan:** I have heard cycle used as though everyone knows what it means. Please explain that too.
- 12 **Dr Bell:** A ⁶ _____ is a repeated unit of the schedule. Its details depend on the actual plan.
- 13 **Jordan:** I would also like to understand the other reasonable options, not just the one named first.
- 14 **Dr Bell:** We should compare each ⁷ _____, including the expected benefits, limitations, and demands relevant to you.
- 15 **Jordan:** Being able to function at home matters to me. A scan is not the only thing I will judge by.
- 16 **Dr Bell:** Your ⁸ _____ is important. We need to understand the outcomes that matter to you.
- 17 **Jordan:** I am not ready to sign anything just because an appointment slot might be available.
- 18 **Dr Bell:** ⁹ _____ is a discussion and a voluntary decision, not simply obtaining a signature.
- 19 **Jordan:** Then let us compare the possible benefit with the burdens before calling this a settled plan.
- 20 **Dr Bell:** Yes. We will review the ¹⁰ _____ and your priorities; no final choice has been made in this conversation.

ANSWERS / EXPLANATIONS: PAGE 92

1. treatment intent | 2. Disease control | 3. caregiving responsibility | 4. Treatment burden | 5. treatment regimen | 6. treatment cycle | 7. alternative option | 8. quality of life | 9. Informed consent | 10. benefit-risk balance

A goal before a schedule

F Say it

Reese asks about a proposed option whose stated aim is symptom relief. The schedule has not been agreed. Reese wants its practical demands explained before deciding.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

agreed / open / relief / demands

Clinician: "The stated aim is symptom ¹ _____."

Reese: "The schedule is not yet ² _____."

Clinician: "We need to explain the practical ³ _____."

Reese: "My decision remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. relief | 2. agreed | 3. demands | 4. open

Rehearse with a fixed script

1. Read the corrected options conversation. Contrast intended benefit with promised outcome.
2. Swap roles. Repeat the caregiving-priority question and the consent explanation without choosing a treatment for Jordan.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The call cannot become a routine message

A Read the situation

05

Give a concise urgent handoff that distinguishes a reported symptom from an established treatment complication.

Oncology nurse Ana calls Dr Ford about Lee, who reports fever and shaking after recent systemic treatment. The urgent oncology assessment pathway has already been activated according to the service protocol. The cause and laboratory results are unknown. The clinicians must exchange the relevant history and confirm responsibility; this case provides no triage threshold, diagnosis, or dosing instruction.

PEOPLE / Ana: Oncology nurse | Dr Ford: Oncologist

URGENCY AND UNCERTAINTY CAN COEXIST

A clinician can request urgent assessment without claiming a confirmed cause. Lead with the concern and action already taken, then identify the information still needed.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is reported?

- | | |
|---|---|
| A A completed response assessment | B No symptoms |
| C Fever and shaking after recent treatment | D A confirmed laboratory diagnosis |

2. What action is already taken?

- | | |
|--|---|
| A All results are normal | B The urgent assessment pathway is activated |
| C A routine message is awaiting next week | D A dose has been selected here |

3. What remains unknown?

- | | |
|---|---------------------------------------|
| A Whether symptoms were reported | B Whether treatment was recent |
| C Cause and laboratory results | D Who called Dr Ford |

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

treatment-related toxicity

Harmful effect attributed to treatment after appropriate assessment.

IN USE assess suspected treatment-related toxicity

febrile episode

Period during which fever occurs.

IN USE report a febrile episode

neutropenia

Abnormally low neutrophil count.

IN USE assess for neutropenia

neutrophil count

Measurement of the number of neutrophils in blood.

IN USE review the neutrophil count

immunosuppression

Reduced activity or effectiveness of immune defenses.

IN USE consider immunosuppression

infection concern

Suspicion that infection may be relevant to a clinical presentation.

IN USE escalate an infection concern

infusion reaction

Adverse response occurring in association with an infusion.

IN USE report a suspected infusion reaction

extravasation

Escape of an infused substance from a vessel into surrounding tissue.

IN USE escalate suspected extravasation

mucositis

Inflammation affecting mucous membranes.

IN USE describe mucositis symptoms

dehydration

Deficit of body water requiring clinical assessment in context.

IN USE assess suspected dehydration

adverse event

Unfavorable occurrence during care, not necessarily caused by treatment.

IN USE report an adverse event

causality assessment

Evaluation of whether one event caused another.

IN USE preserve uncertainty in causality assessment

Precision check

Fever after treatment raises a concern requiring the actual clinical pathway; it does not establish neutropenia, infection, or treatment causality by itself.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

symptom onset time

Time when the reported symptom began.

IN USE confirm symptom onset time

last treatment date

Most recent date on which treatment was administered.

IN USE verify the last treatment date

urgent assessment

Prompt clinical evaluation through the appropriate service pathway.

IN USE arrange urgent assessment

receiving clinician

Professional taking responsibility for the next clinical assessment.

IN USE identify the receiving clinician

read-back

Repetition of important information to verify its accuracy.

IN USE use read-back for key details

escalation acknowledgment

Confirmation that an urgent request has been received and accepted.

IN USE obtain escalation acknowledgment

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Do not delay an urgent response to finish a language exercise or obtain a perfect history. This scenario begins after the real service pathway has been activated.

Phrases you can actually use

C Notice the language / 1 of 2

LEAD WITH URGENCY

"I am calling about an urgent symptom report after recent treatment."

ATTRIBUTE THE REPORT

"Lee reports fever and shaking."

AVOID PREMATURE DIAGNOSIS

"I am reporting the concern, not a confirmed cause."

READ BACK

"I will repeat the key details to check accuracy."

STATE THE ACTION

"The urgent assessment pathway is already activated."

PRESERVE AN UNKNOWN

"We do not yet have the laboratory results."

CONFIRM RESPONSIBILITY

"Who is accepting the next assessment?"

SEPARATE PROCESSES

"This must not be left as a routine administrative message."

Reports versus has confirmed

Separates the patient's symptom account from an established clinical diagnosis.

Already activated

Marks an action completed before this handoff begins.

Accepting

Makes the transfer of responsibility explicit rather than implying that a message sent itself completes a handoff.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening best supports the handoff?

- A** There is a small administrative question for next week.
- B** All causes are already established.
- C** Lee must be fine because the call is clear.
- D** Urgent symptom report after recent treatment; the pathway is activated.

2. Which diagnosis cannot be asserted from this brief?

- A** Confirmed neutropenia
- B** Reported fever
- C** Reported shaking
- D** Recent treatment

3. What must the handoff confirm?

- A** Only that an email was sent
- B** An invented medicine dose
- C** A guaranteed cause
- D** The receiving clinician and responsibility

4. Which statement preserves causality correctly?

- A** Treatment definitely caused every symptom.
- B** Timing proves infection.
- C** No assessment is needed until causality is certain.
- D** Symptoms occurred after treatment; the cause is not established.

Answers and explanations: [page 93](#).

The call cannot become a routine message

E Conversations / turns 1-10

05

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

read-back / infection concern / neutrophil count / adverse event / causality assessment / last treatment date / symptom onset time / urgent assessment / escalation acknowledgment / receiving clinician

- 01 **Ana:** I am calling about Lee, who reports fever and shaking after recent systemic treatment. The urgent pathway is activated.
- 02 **Dr Ford:** Thank you. Keep this with ¹ _____, not a routine message. Tell me the verified details and what remains unknown.
- 03 **Ana:** The symptoms are patient-reported. I am confirming their timing, and we do not yet have laboratory results.
- 04 **Dr Ford:** Please preserve the ² _____ as reported, including any uncertainty in the account.
- 05 **Ana:** I will also verify the most recent treatment information from the actual record rather than rely on memory.
- 06 **Dr Ford:** Yes. The ³ _____ matters, but do not delay the active response while completing every detail.
- 07 **Ana:** Shall I describe this as confirmed treatment toxicity in the handoff heading, or keep the cause open?
- 08 **Dr Ford:** Keep the ⁴ _____ open. The symptoms followed treatment, but we have not established their cause.
- 09 **Ana:** I have not labeled Lee neutropenic. There is no count available to support that description yet.
- 10 **Dr Ford:** Correct. A ⁵ _____ is not available; do not fill that gap with an assumed result.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Ana:** The clinical concern remains urgent even while we avoid assigning a diagnosis that has not been established.
- 12 **Dr Ford:** Exactly. We can escalate an ⁶ _____ without declaring that infection is confirmed.
- 13 **Ana:** I need a clear receiving contact so that the report does not move between teams without an owner.
- 14 **Dr Ford:** The ⁷ _____ must be identified through our pathway. I am accepting this clinical discussion now.
- 15 **Ana:** I will repeat the patient's reported symptoms and the actions already taken, then confirm the next communication.
- 16 **Dr Ford:** Use ⁸ _____ for the important details and correct anything that differs from the record.
- 17 **Ana:** I will document that the concern was received and who accepted it, not simply that a call was attempted.
- 18 **Dr Ford:** That ⁹ _____ makes the handoff clear. Keep any new information with the active clinical team.
- 19 **Ana:** Understood. The urgent pathway remains active, the cause is unresolved, and we are not inventing missing results.
- 20 **Dr Ford:** Correct. Record the ¹⁰ _____ factually and follow the actual clinical and reporting procedures.

ANSWERS / EXPLANATIONS: PAGE 94

1. urgent assessment | 2. symptom onset time | 3. last treatment date | 4. causality assessment | 5. neutrophil count | 6. infection concern | 7. receiving clinician | 8. read-back | 9. escalation acknowledgment | 10. adverse event

Confirm a received escalation

F Say it

The urgent pathway is active for a new symptom report. Dr Lane accepts the handoff. Laboratory results remain pending; the cause is not established.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

established / active / pending / accept

Nurse: "The urgent pathway is ¹ _____."

Dr Lane: "I ² _____ the handoff."

Nurse: "The laboratory results remain ³ _____."

Dr Lane: "The cause is not ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. active | 2. accept | 3. pending | 4. established

Rehearse with a fixed script

1. Read the corrected urgent handoff. Lead with the concern and the pathway already activated.
2. Swap roles. Repeat the unknown-results statement and the acknowledgment without inventing a diagnosis or a dose.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Smaller is useful, but not the whole conclusion

A Read the situation

Explain a scan comparison while distinguishing measurement, interpretation, and an agreed treatment decision.

06

Pat's report describes a measured lesion as smaller than on the comparison scan. The complete response assessment is still under oncology review, and no formal response category or treatment change has been agreed. Dr Ng must acknowledge the encouraging observation without declaring cure, complete response, or a settled next regimen.

PEOPLE / Pat: Patient | Dr Ng: Oncologist

SHARE THE USEFUL FINDING WITHOUT EXPANDING ITS MEANING

Patients deserve to hear favorable observations clearly. Precision does not require withholding them; it requires separating the observation from conclusions still under review.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the report describe?

A A confirmed cure

C A measured lesion is smaller

B A new regimen already started

D Every lesion has disappeared

2. What is still under review?

A Whether Pat has a report

C Whether a clinician is involved

B Whether a comparison scan exists

D The complete response assessment

3. What has not been agreed?

A That the lesion was measured

C A treatment change

B That Pat wants an explanation

D That the report is available

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

response assessment

Evaluation of how disease has changed during or after treatment.

IN USE complete a response assessment

comparison scan

Earlier imaging study used as a reference.

IN USE review the comparison scan

measurable lesion

Abnormality that can be measured under the relevant assessment criteria.

IN USE identify a measurable lesion

target lesion

Selected lesion followed for measurement under a response-assessment system.

IN USE measure a target lesion

non-target lesion

Disease finding assessed outside the selected target-lesion measurements.

IN USE assess non-target lesions

partial response

Reduction meeting the relevant criteria without a complete response.

IN USE explain a partial response

complete response

Disappearance of detectable disease findings under the applicable response criteria.

IN USE qualify a complete response

stable disease

Disease not meeting the relevant response or progression thresholds.

IN USE discuss stable disease

progressive disease

Worsening that meets applicable progression criteria.

IN USE evaluate progressive disease

new lesion

Newly identified abnormality requiring interpretation in context.

IN USE investigate a new lesion

radiological response

Change in disease appearance assessed using imaging.

IN USE describe radiological response

clinical response

Change assessed through clinical symptoms, signs, or functioning.

IN USE compare clinical response

Precision check

A smaller lesion is an observation, not automatically a formal partial response. Complete response and cure are not interchangeable claims.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

remission

Reduction or disappearance of signs of disease, described as partial or complete when appropriate.

IN USE explain remission

recurrence

Return of cancer after a period of improvement or remission.

IN USE discuss recurrence

measurement variability

Differences arising from how measurements are obtained or interpreted.

IN USE account for measurement variability

scan interval

Time between imaging examinations.

IN USE confirm the scan interval

restaging

Reassessment of disease extent after treatment or a change.

IN USE explain restaging

assessment criteria

Defined rules used to classify findings or changes.

IN USE apply the relevant assessment criteria

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Use the applicable cancer-specific assessment criteria and full clinical context. The exercise supplies no measurement thresholds and cannot classify a real scan.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE OBSERVATION

"This measured lesion is smaller than on the comparison scan."

SEPARATE THE REVIEW

"The complete response assessment is still under review."

INCLUDE OTHER EVIDENCE

"The team will interpret the scan with the other relevant findings."

EXPLAIN A CATEGORY

"A response category uses defined criteria, not one adjective."

RECOGNIZE ITS VALUE

"That is a useful finding to discuss."

AVOID OVERSTATEMENT

"Smaller does not by itself mean cured."

KEEP STATUS ACCURATE

"No treatment change has been agreed yet."

CHECK THE SUMMARY

"What would you tell someone about what is known and what is still pending?"

06

Smaller than

Makes a comparative observation and identifies the reference image.

By itself

Limits the conclusion supported by one finding.

Has been agreed

Distinguishes an agreed decision from a possible change under discussion.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

06

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which update is supported?

- A** One measured lesion is smaller; the full assessment remains under review.
- B** Pat is cured.
- C** Complete response is confirmed.
- D** Every finding has resolved.

2. Which claim confuses observation and classification?

- A** A comparison scan is used.
- B** A lesion was measured.
- C** The review is incomplete.
- D** Smaller automatically means formal partial response.

3. Which treatment statement is accurate?

- A** No change has been agreed.
- B** A new regimen is already authorized.
- C** Treatment has ended permanently.
- D** The next dose is supplied here.

4. Why avoid equating complete response with cure?

- A** Complete response means no review is needed.
- B** They describe different claims about disease and future outcome.
- C** The words have identical meanings.
- D** Cure is merely a measurement unit.

Answers and explanations: [page 95](#).

Smaller is useful, but not the whole conclusion

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

06

WORD + PHRASE BANK

target lesion / complete response / response assessment / assessment criteria / Remission / Non-target lesions / partial response / radiological response / Clinical response / comparison scan

- 01 **Pat:** The report says the spot is smaller. I have been waiting to hear whether I can call that good news.
- 02 **Dr Ng:** The ¹ _____ shows the reference point. This measured lesion is smaller, and that is useful information.
- 03 **Pat:** I would like to understand the finding without saying more than the report can actually support.
- 04 **Dr Ng:** That is sensible. The full ² _____ is still under review; one measurement is not the entire conclusion.
- 05 **Pat:** Does smaller automatically mean the treatment has produced what the team calls a partial response?
- 06 **Dr Ng:** A ³ _____ has defined criteria. I need the full assessment before using that label for your results.
- 07 **Pat:** I noticed the report described other findings as well. Are they part of the same review?
- 08 **Dr Ng:** Yes. A ⁴ _____ is selected for measurement, but other findings can matter to the overall interpretation.
- 09 **Pat:** Then I should not assume that measuring one selected area means every other area has been ignored.
- 10 **Dr Ng:** Correct. ⁵ _____ and any new findings may need assessment under the appropriate system.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Pat:** My symptoms have also changed. Should I describe those even if the scan appears encouraging?
- 12 **Dr Ng:** Absolutely. ⁶ _____ and imaging findings offer different information; we need the clinical context too.
- 13 **Pat:** I want to tell my family there is an encouraging change, but not that everything has disappeared.
- 14 **Dr Ng:** That is accurate. A ⁷ _____ has not been established by the facts we are discussing.
- 15 **Pat:** And even that term would need explaining rather than being used as a simple synonym for cure?
- 16 **Dr Ng:** Yes. ⁸ _____ and cure are not interchangeable promises about an individual's future.
- 17 **Pat:** Does the smaller measurement mean you have already decided to change the treatment schedule?
- 18 **Dr Ng:** No. We must finish the review using the relevant ⁹ _____ and clinical context before discussing any recommendation.
- 19 **Pat:** Then my summary is that one lesion is smaller, the full review is pending, and the plan has not changed yet.
- 20 **Dr Ng:** Exactly. We will explain the ¹⁰ _____ and its implications when the complete assessment is available.

06

ANSWERS / EXPLANATIONS: PAGE 96

1. comparison scan | 2. response assessment | 3. partial response | 4. target lesion | 5. Non-target lesions | 6. Clinical response | 7. complete response | 8. Remission | 9. assessment criteria | 10. radiological response

A new finding awaiting interpretation

F Say it

A scan mentions a new finding. The oncology review is incomplete. No progression category or treatment change has been agreed.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

agreed / classified / new / incomplete

Patient: "The report describes a ¹ _____ finding."

Clinician: "The review remains ² _____."

Patient: "Progression is not yet ³ _____."

Clinician: "No treatment change is ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. new | 2. incomplete | 3. classified | 4. agreed

Rehearse with a fixed script

1. Read the corrected scan discussion. Keep the encouraging observation separate from a formal response category.
2. Swap roles. Repeat Pat's final summary without declaring cure or changing treatment.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

An option to discuss, not an obligation

A Read the situation

Explain research participation while separating eligibility, consent, and proven benefit.

Dr Ortiz discusses a possible treatment trial with Val. Eligibility screening is not complete, and Val has not consented or enrolled. The trial compares approaches using random assignment. No investigational product, benefit probability, or payment terms are supplied. Val worries that declining would harm the relationship with the care team.

07

PEOPLE / Val: Patient | Dr Ortiz: Oncologist

KEEP RESEARCH AND CARE ROLES CLEAR

An invitation can feel like a recommendation the patient must obey. Explain the research purpose, uncertainty, and voluntary decision without presenting an experimental option as proven superior.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the participation status?

A Already enrolled

C Benefit already demonstrated for Val

B Assigned to a study group

D Eligibility review incomplete; no consent or enrollment

2. How does the trial assign approaches?

A A payment determining the group

C Val choosing any group after enrollment

B Random assignment

D A guaranteed best-treatment allocation

3. What worries Val?

A A completed treatment switch

C A dose already selected here

B Declining might harm the care relationship

D A result already received

Answers and explanations: [page 97](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

clinical trial

Research study evaluating a health intervention in people.

IN USE discuss a clinical trial

eligibility criteria

Requirements determining whether a person may enter a study.

IN USE review eligibility criteria

screening visit

Assessment checking whether trial-entry requirements are met.

IN USE arrange a screening visit

enrollment

Formal entry into a study after required processes.

IN USE distinguish enrollment from screening

randomization

Assignment to study groups through a chance-based process.

IN USE explain randomization

study arm

Group receiving a specified intervention or approach in a trial.

IN USE describe a study arm

control group

Comparison group used to evaluate an intervention.

IN USE explain the control group

investigational treatment

Treatment being studied for the relevant use.

IN USE discuss an investigational treatment

standard of care

Accepted clinical approach for the relevant setting and circumstances.

IN USE compare with standard of care

research protocol

Approved plan specifying how a study is conducted.

IN USE explain the research protocol

primary endpoint

Main outcome used to evaluate the study question.

IN USE identify the primary endpoint

overall survival

Time from a defined starting point until death from any cause.

IN USE interpret overall survival

Precision check

Trial eligibility is not enrollment, and consent does not guarantee benefit. Randomization is not a clinician choosing the group believed best for one participant.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

progression-free survival

Time from a defined point without disease progression or death under study definitions.

IN USE explain progression-free survival

voluntary participation

Taking part by choice without improper pressure.

IN USE protect voluntary participation

withdrawal

Ending participation through the applicable study process.

IN USE explain withdrawal options

consent discussion

Conversation supporting an informed decision about participation.

IN USE allow time for the consent discussion

research coordinator

Professional organizing study visits and participation processes.

IN USE contact the research coordinator

therapeutic misconception

Mistaken belief that research is designed solely to provide individualized treatment benefit.

IN USE address therapeutic misconception

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Explain withdrawal and remaining care using the actual protocol and applicable duties. Do not invent payment, access, or administrative guarantees.

Phrases you can actually use

C Notice the language / 1 of 2

INTRODUCE RESEARCH

"This is an option to discuss, not an obligation."

EXPLAIN ASSIGNMENT

"Random assignment determines the study group."

PROTECT THE RELATIONSHIP

"Declining research does not remove your right to appropriate care."

AVOID A GUARANTEE

"An investigational treatment is not proven better simply because it is new."

STATE UNCERTAINTY

"The study is asking a question that is not already settled."

SEPARATE STAGES

"Eligibility review is not the same as enrollment."

INVITE REVIEW

"Take the opportunity to ask about visits, risks, and alternatives."

CLARIFY WITHDRAWAL

"We will explain the actual withdrawal process and remaining care arrangements."

May be eligible

Describes a possibility before the entry criteria have been checked.

Investigational

Identifies a research status, not a promise of superior treatment.

Not an obligation

Removes an implied demand while leaving the option available for discussion.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement separates the stages correctly?

- A** Discussing a trial automatically enrolls Val.
- B** Screening proves benefit.
- C** A consent form guarantees entry.
- D** Eligibility is under review; Val is not enrolled.

2. Which claim creates therapeutic misconception?

- A** Participation is voluntary.
- B** Random assignment determines a group.
- C** The research exists solely to give Val the best individual treatment.
- D** The study has a research question.

3. What does randomization mean?

- A** A completed response assessment
- B** Chance-based assignment under the study process
- C** The doctor selecting a guaranteed best arm
- D** The patient purchasing a preferred arm

4. Which response to Val's concern is appropriate?

- A** Signing now is the only way to ask questions.
- B** Declining research does not remove the right to appropriate care.
- C** The team will stop listening if you decline.
- D** You owe the study your participation.

Answers and explanations: [page 97](#).

An option to discuss, not an obligation

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

consent discussion / clinical trial / withdrawal / investigational treatment / eligibility criteria / research coordinator / standard of care / Enrollment / research protocol / randomization

07

- 01 **Val:** You mentioned a trial. I worry that saying no would make you think I am not cooperating with care.
- 02 **Dr Ortiz:** This ¹ _____ is an option to discuss. Declining research does not remove your right to appropriate care.
- 03 **Val:** Am I already accepted because you brought it up, or is there another process before that?
- 04 **Dr Ortiz:** We still need to review the ² _____. Discussing the study does not establish that you can enter it.
- 05 **Val:** I would like to understand what the study is trying to find out before considering any forms.
- 06 **Dr Ortiz:** The ³ _____ describes its question, procedures, and participant requirements. We will review the relevant details with you.
- 07 **Val:** Would you choose whichever treatment group you think is best for me after I join?
- 08 **Dr Ortiz:** This study uses ⁴ _____. Assignment follows a chance-based process rather than my choosing an arm for you.
- 09 **Val:** That matters to my decision. I do not want to assume that the new option is already known to be better.
- 10 **Dr Ortiz:** An ⁵ _____ is being studied; its possible advantages and disadvantages are not a promised result for you.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Val:** Please compare participation with the ordinary care options I could discuss outside the study.
- 12 **Dr Ortiz:** We should explain the relevant ⁶ _____ and other reasonable alternatives, not present research as the only possible conversation.
- 13 **Val:** The visits and tests may affect my work. Can I ask about those before deciding whether to proceed?
- 14 **Dr Ortiz:** Certainly. The ⁷ _____ can help explain practical arrangements, alongside our discussion of the clinical questions.
- 15 **Val:** If I first agree and later want to stop, I need to understand what happens to my care.
- 16 **Dr Ortiz:** We will explain ⁸ _____ using the actual study process and clarify the remaining care arrangements.
- 17 **Val:** I would like time to read the information and return with questions rather than sign because I feel rushed.
- 18 **Dr Ortiz:** That is part of a proper ⁹ _____. Questions and time to understand are important to a voluntary decision.
- 19 **Val:** Then eligibility is still being checked, and I have not consented or joined by having this conversation.
- 20 **Dr Ortiz:** Correct. ¹⁰ _____ has not happened. We can continue discussing the option without treating today as a commitment.

ANSWERS / EXPLANATIONS: PAGE 98

1. clinical trial | 2. eligibility criteria | 3. research protocol | 4. randomization | 5. investigational treatment | 6. standard of care | 7. research coordinator | 8. withdrawal | 9. consent discussion | 10. Enrollment

A screening visit is not enrollment

F Say it

Jo has agreed to a trial-screening visit. Eligibility remains unknown. Jo has not enrolled and wants the withdrawal information explained before making a participation decision.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

enrolled / withdrawal / screening / unknown

Jo: "I agreed to a ¹ _____ visit."

Clinician: "Your eligibility remains ² _____."

Jo: "I have not ³ _____ in the study."

Clinician: "We will explain the ⁴ _____ process."

ANSWERS / EXPLANATIONS: PAGE 98

1. screening | 2. unknown | 3. enrolled | 4. withdrawal

Rehearse with a fixed script

1. Read the corrected research discussion. Contrast invitation, eligibility, consent, and enrollment.
2. Swap roles. Repeat the randomization explanation and the response to perceived pressure.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Support alongside treatment

A Read the situation

Explain a supportive-care referral and elicit goals without equating it with abandonment or an automatic change in treatment.

Dr Ahmed offers Rowan a palliative-care consultation for symptoms and distress while an anticancer-treatment discussion continues. Rowan hears the referral as a decision to stop treatment. No decision to stop treatment, enter hospice, or set a resuscitation status is supplied. Rowan wants more comfortable time with family and clearer help with symptoms.

PEOPLE / Rowan: Patient | Dr Ahmed: Oncologist

08

EXPLORE THE MEANING THE PATIENT HEARD

Terms associated with serious illness can carry meanings beyond their clinical definition. Ask what the referral sounded like before correcting the misunderstanding.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Why is the consultation offered?

- A A completed resuscitation decision
- B Symptoms and distress
- C A supplied decision to stop all treatment
- D An automatic hospice admission

2. What does Rowan fear?

- A That a trial has enrolled them
- B That treatment is being stopped
- C That the report contains a new grade
- D That a scan is already normal

3. Which preference is stated?

- A More comfortable time with family
- B A particular resuscitation status
- C A guaranteed survival period
- D An instruction to refuse all services

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

palliative care

Care addressing symptoms, distress, and quality of life in serious illness.

IN USE introduce palliative care

hospice care

Care focused on comfort near the end of life under applicable service criteria.

IN USE distinguish hospice care

symptom management

Assessment and treatment of distressing symptoms.

IN USE coordinate symptom management

goals-of-care discussion

Conversation about priorities and how care should respond to them.

IN USE open a goals-of-care discussion

serious-illness conversation

Discussion of understanding, concerns, values, and care in serious illness.

IN USE structure a serious-illness conversation

advance care planning

Process of discussing and recording preferences for future care.

IN USE offer advance care planning

advance directive

Document expressing future-care wishes or decision arrangements under applicable law.

IN USE review an advance directive

surrogate decision-maker

Person authorized to decide when the patient cannot under applicable rules.

IN USE clarify the surrogate decision-maker

resuscitation status

Documented decision or order about resuscitation in the relevant setting.

IN USE clarify resuscitation status

comfort-focused care

Care organized primarily around comfort and relief of distress.

IN USE explain comfort-focused care

concurrent care

Care approaches provided alongside one another.

IN USE discuss concurrent care

care preference

Person's expressed wishes about aspects of care.

IN USE elicit a care preference

08

Precision check

Palliative care can accompany anticancer treatment. A referral does not automatically mean hospice enrollment, treatment cessation, or a resuscitation decision.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

family meeting

Planned discussion involving the patient and appropriate family or support people.

IN USE arrange a family meeting

caregiver strain

Physical or emotional burden experienced by someone providing care.

IN USE acknowledge caregiver strain

spiritual concern

Concern about meaning, belief, connection, or existential distress.

IN USE invite a spiritual concern

symptom burden

Combined effect of symptoms on the person's life.

IN USE assess symptom burden

interdisciplinary support

Help provided by professionals with complementary care roles.

IN USE coordinate interdisciplinary support

values clarification

Exploration of what matters to a person in a care decision.

IN USE support values clarification

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A family member's presence does not automatically establish decision-making authority. Discuss preferences and legal arrangements accurately for the setting.

Phrases you can actually use

C Notice the language / 1 of 2

ASK WHAT WAS HEARD

"What did the phrase palliative care suggest to you?"

EXPLAIN THE PURPOSE

"The team can help with symptoms and distress alongside other care."

SEPARATE DECISIONS

"Hospice and resuscitation decisions require their own discussions."

AVOID ABANDONMENT

"We will continue to address your care needs."

CORRECT THE INFERENCE

"This referral is not a decision to stop treatment."

ELICIT A PRIORITY

"What would make the coming weeks more manageable for you?"

INVITE SUPPORT

"We can discuss who you want involved in a family meeting."

MAKE GOALS CONCRETE

"More comfortable time with family is a priority we should keep visible."

08

Alongside

Describes concurrent care rather than replacement or abandonment.

Not a decision to

Corrects an inference about an action that has not been agreed.

What would make

Invites a concrete priority rather than assuming the clinician knows it already.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement explains the referral correctly?

- A** Palliative care can support symptoms alongside treatment.
- B** The referral automatically stops anticancer treatment.
- C** The referral determines resuscitation status.
- D** The referral guarantees hospice admission.

2. Which question explores the misunderstanding?

- A** Why did you misunderstand an obvious term?
- B** You agree it means giving up?
- C** Can we avoid your concern?
- D** What did the phrase suggest to you?

3. What priority should remain visible?

- A** Comfortable time with family
- B** A resuscitation choice invented here
- C** A guaranteed prognosis
- D** A decision attributed to an absent relative

4. Which assumption is unsafe?

- A** Preferences need discussion.
- B** Symptoms need attention.
- C** Different care decisions should be separated.
- D** Every relative present has automatic decision-making authority.

Answers and explanations: [page 99](#).

Support alongside treatment

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

goals-of-care discussion / family meeting / palliative care / surrogate decision-maker / symptom burden / Hospice care / symptom management / concurrent care / resuscitation status / Caregiver strain

08

- 01 **Rowan:** When you mentioned palliative care, I heard that you were giving up on the cancer treatment.
- 02 **Dr Ahmed:** What did ¹ _____ suggest to you? I want to understand that concern before explaining the referral.
- 03 **Rowan:** I thought it meant every other treatment would stop, and that I would no longer see this team.
- 04 **Dr Ahmed:** This is an offer of ² _____. It is not a decision to stop treatment or to stop addressing your care needs.
- 05 **Rowan:** Then please tell me what the additional team would actually help with in everyday life.
- 06 **Dr Ahmed:** They can help with ³ _____, distress, and the effects of serious illness on you and those supporting you.
- 07 **Rowan:** I want more comfortable time with my family. At present the symptoms take over the entire day.
- 08 **Dr Ahmed:** That ⁴ _____ matters. We should keep comfortable family time visible when we discuss care options.
- 09 **Rowan:** Does agreeing to this consultation mean I have agreed to hospice without realizing it?
- 10 **Dr Ahmed:** No. ⁵ _____ is a separate discussion with its own circumstances and service criteria.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Rowan:** I also do not want one referral to become a decision about resuscitation without anyone asking me.
- 12 **Dr Ahmed:** It does not establish your ⁶ _____. Those decisions require their own appropriate discussion and documentation.
- 13 **Rowan:** My daughter helps a great deal, but I do not want everyone to assume she makes all decisions for me.
- 14 **Dr Ahmed:** We should clarify that. A ⁷ _____ has a specific role under the applicable arrangements, not simply because a relative attends.
- 15 **Rowan:** Could we have a conversation together where she can hear what I want without taking over the discussion?
- 16 **Dr Ahmed:** We can discuss a ⁸ _____ and who you want involved, keeping your priorities and permissions clear.
- 17 **Rowan:** I also want someone to notice how exhausted she is. She keeps saying she is fine.
- 18 **Dr Ahmed:** ⁹ _____ is worth discussing, with her involvement and the appropriate support.
- 19 **Rowan:** I understand better now. The referral adds symptom support; it does not silently settle every future decision.
- 20 **Dr Ahmed:** Exactly. A ¹⁰ _____ keeps those priorities explicit while we continue the other necessary clinical conversations.

ANSWERS / EXPLANATIONS: PAGE 100

1. palliative care | 2. concurrent care | 3. symptom management | 4. symptom burden | 5. Hospice care | 6. resuscitation status | 7. surrogate decision-maker | 8. family meeting | 9. Caregiver strain | 10. goals-of-care discussion

Name a concrete care priority

F Say it

Taylor wants symptom support while treatment options are reviewed. Taylor would like friend Jo at a meeting but has not named Jo as a legally authorized decision-maker.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

review / symptoms / authority / Jo

08

Taylor: "I want help with my ¹ _____."

Clinician: "Treatment options remain under ² _____."

Taylor: "I would like ³ _____ at the meeting."

Clinician: "Attendance does not establish decision-making ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. symptoms | 2. review | 3. Jo | 4. authority

Rehearse with a fixed script

1. Read the corrected supportive-care conversation. Stress alongside and separate discussion.
2. Swap roles. Repeat the response to Rowan's fear without promising an outcome or assigning a resuscitation status.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Preparing a second-opinion record

E More workplace conversations / 1-10

Support a second opinion and distinguish a complete referral packet from a guaranteed recommendation.

Patient Mira asks Dr Hart for a second opinion. The receiving service requests the pathology report, relevant imaging, and treatment summary, with authorized transfer arrangements. The clinician supports the request. No new treatment recommendation or review appointment is confirmed.

PEOPLE / Mira: Patient | Dr Hart: Oncologist

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

referral packet / treatment summary / imaging studies / record-transfer authorization /
Appointment confirmation / pathology report

- 01 **Mira:** I would like a second opinion, but I worry that asking will sound as though I do not trust you.
- 02 **Dr Hart:** You can ask for another review. Let us discuss the information the receiving service needs and the transfer process.
- 03 **Mira:** They requested a pathology report. Is that different from the short diagnosis line in my appointment letter?
- 04 **Dr Hart:** Yes. The ¹ _____ contains the relevant findings, not merely the abbreviated label in a letter.
- 05 **Mira:** They also asked for the images, not only the written scan report. Why would both be requested?
- 06 **Dr Hart:** The reviewing team may need the actual ² _____ as well as the reports describing them.
- 07 **Mira:** I have had treatment at two sites. I do not want the second team to assume the list from one site is complete.
- 08 **Dr Hart:** We should verify the ³ _____ across the relevant records, including what remains unconfirmed.
- 09 **Mira:** Does sending all the records mean the second team will definitely recommend something different?
- 10 **Dr Hart:** No. A second review may agree, differ, or need further information; we cannot promise its conclusion.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Mira:** I would like to know what I am authorizing when the records are transferred.
- 12 **Dr Hart:** The ⁴ _____ should identify the relevant information and recipient through the approved process.
- 13 **Mira:** If a specimen is requested for review, is that automatically included with an electronic report?
- 14 **Dr Hart:** Not necessarily. Any specimen or slide request has its own actual handling and transfer arrangements.
- 15 **Mira:** I will ask the receiving service what is still missing instead of assuming that one uploaded file completes everything.
- 16 **Dr Hart:** That is useful. A ⁵ _____ can be incomplete even when some documents have arrived.
- 17 **Mira:** Who will tell me whether the review appointment is actually confirmed?
- 18 **Dr Hart:** We should identify the receiving contact and distinguish receipt of records from an accepted appointment.
- 19 **Mira:** Then I can ask for a review without treating the transfer itself as a new treatment plan.
- 20 **Dr Hart:** Exactly. ⁶ _____ and the eventual clinical recommendation remain separate steps.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. pathology report | 2. imaging studies | 3. treatment summary | 4. record-transfer authorization | 5. referral packet | 6. Appointment confirmation

The images have not arrived

F Say it in a new situation

The receiving team has the pathology report and treatment summary but not the imaging studies. No appointment is confirmed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

missing / complete / arrived / confirmed

Patient: "The imaging studies are still ¹ _____."

Clinician: "The report and summary have ² _____."

Patient: "The referral packet is not ³ _____."

Clinician: "The appointment is not yet ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 101

1. missing | 2. arrived | 3. complete | 4. confirmed

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

An oncology handoff after hospital discharge

E More workplace conversations / 1-10

Reconcile inpatient changes with the oncology plan and assign unresolved questions.

Hospital clinician Dr Sen hands over patient Jules to oncologist Dr Ford after discharge. A medicine list was changed in hospital, but the outpatient record still shows an earlier list. The clinicians will verify the actual discharge instructions and assign follow-up. No medicine names, doses, or authority to restart treatment are supplied.

PEOPLE / Dr Sen: Hospital clinician | Dr Ford: Oncologist

Use each bank entry once, without changing its form. Answers follow turn 20.

10

WORD + PHRASE BANK

medication discrepancy / handoff acknowledgment / discharge summary / treatment hold / result ownership / reconciled plan

- 01 **Dr Sen:** Jules has been discharged, and the hospital medicine list differs from the earlier outpatient list. We need to reconcile them.
- 02 **Dr Ford:** Please identify the verified changes and their source rather than asking the patient to choose between two conflicting lists.
- 03 **Dr Sen:** The ¹ _____ is available, but I want to confirm that the current version reached your team.
- 04 **Dr Ford:** We will acknowledge receipt and review the actual instructions before presenting the earlier list as current.
- 05 **Dr Sen:** Some changes relate to the hospital episode, and the outpatient record has not yet been updated.
- 06 **Dr Ford:** That creates a ² _____ requiring verification. The language of the handoff must not imply that both lists are simultaneously correct.
- 07 **Dr Sen:** I will distinguish a documented instruction from a suggestion that still needs the responsible clinician's decision.
- 08 **Dr Ford:** Good. We also need the reason for each verified change, where the actual record provides it.
- 09 **Dr Sen:** A note mentions treatment being held, but the wording alone does not authorize restarting it today.
- 10 **Dr Ford:** A ³ _____ and a restart decision are different. We need the responsible oncology assessment.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Dr Sen:** The patient is worried that discharge means every cancer-care question has already been settled.
- 12 **Dr Ford:** Discharge does not automatically resolve the outpatient plan. We should explain what is confirmed and what remains open.
- 13 **Dr Sen:** One result is still pending, and its reviewing clinician needs to be identified explicitly.
- 14 **Dr Ford:** Let us assign ⁴ _____ instead of assuming that whichever team sees it first will take responsibility.
- 15 **Dr Sen:** I will include the appropriate contact for questions about the hospital changes.
- 16 **Dr Ford:** And our team will confirm the oncology follow-up arrangements through the actual service process.
- 17 **Dr Sen:** The patient should receive one clear explanation once the conflicting information is resolved.
- 18 **Dr Ford:** A ⁵ _____ needs verified instructions, not a merged list that preserves contradictions.
- 19 **Dr Sen:** I will send the current summary and confirm that the outstanding tasks have named owners.
- 20 **Dr Ford:** We will provide ⁶ _____, keeping discharge, pending results, and treatment decisions distinct.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. discharge summary | 2. medication discrepancy | 3. treatment hold | 4. result ownership | 5. reconciled plan | 6. handoff acknowledgment

A pending result needs an owner

F Say it in a new situation

The discharge summary lists a pending result but no reviewing clinician. The two teams clarify who will review it. No result is yet available.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

named / pending / result / ownership

Hospital clinician: "The result is still ¹ _____."

Oncologist: "The reviewing clinician was not ² _____."

Hospital clinician: "We need explicit ³ _____."

Oncologist: "No conclusion can be drawn from an unavailable ⁴ _____."

10

ANSWERS / EXPLANATIONS: PAGE 102

1. pending | 2. named | 3. ownership | 4. result

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Discussing the financial burden of treatment

E More workplace conversations / 1-10

Invite practical concerns without promising coverage or treating cost-related difficulty as refusal.

Patient Rowan tells Dr Bell that transport costs and unpaid time away from work may interfere with appointments. Dr Bell asks permission to involve the support team and discusses verifying actual coverage and assistance options. No insurance approval, eligibility decision, or treatment substitution is supplied.

PEOPLE / Rowan: Patient | Dr Bell: Oncologist

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

cost-related barrier / Financial toxicity / treatment tradeoff / patient navigation / out-of-pocket costs / Eligibility review

11

- 01 **Rowan:** I want to keep my appointments, but transport and unpaid time away from work are becoming difficult to manage.
- 02 **Dr Bell:** Thank you for telling me. Those barriers belong in the care discussion, not in a judgment about your commitment.
- 03 **Rowan:** I was afraid that mentioning money would be recorded as refusing the treatment.
- 04 **Dr Bell:** A ¹ _____ is different from refusing treatment. We should document what is actually making attendance difficult.
- 05 **Rowan:** The hospital estimate also uses terms I do not understand, and I do not know which amount I might owe.
- 06 **Dr Bell:** The appropriate team can help clarify ² _____ under your actual coverage, without promising a figure before verification.
- 07 **Rowan:** Could someone help with transport or finding out whether an assistance program is relevant?
- 08 **Dr Bell:** With your permission, we can involve ³ _____ or the relevant support service to explore those options.
- 09 **Rowan:** I would like that, but I do not want my employer contacted automatically.
- 10 **Dr Bell:** We should agree on the information-sharing permissions and recipients rather than treat a support referral as unlimited consent.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Rowan:** If an assistance application is submitted, does that mean the cost will definitely be covered?
- 12 **Dr Bell:** No. ⁴ _____ and approval remain separate steps, and we should be clear about what is still pending.
- 13 **Rowan:** I can provide the documents the team actually needs, but I may need help understanding the forms.
- 14 **Dr Bell:** We can ask what language and administrative support is available through the real service.
- 15 **Rowan:** Does telling you about the cost mean you will change treatment without discussing it with me?
- 16 **Dr Bell:** No. A ⁵ _____ requires a proper clinical discussion; no substitution is being made in this conversation.
- 17 **Rowan:** I would feel better with one contact who can explain what has been applied for and what is confirmed.
- 18 **Dr Bell:** Let us identify that contact and keep applications, estimates, and confirmed arrangements clearly distinguished.
- 19 **Rowan:** Then the practical difficulty can be addressed without assuming that I have chosen to stop care.
- 20 **Dr Bell:** Exactly. ⁶ _____ deserves attention, with honest limits on what we can confirm and a clear next step.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. cost-related barrier | 2. out-of-pocket costs | 3. patient navigation | 4. Eligibility review | 5. treatment tradeoff
| 6. Financial toxicity

An assistance application is pending

F Say it in a new situation

The support team submits an assistance application. Eligibility and coverage are not yet confirmed. The patient has authorized contact with the program, not the employer.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

employer / unconfirmed / authorized / submitted

Patient: "The application has been ¹ _____."

Navigator: "Eligibility remains ² _____."

Patient: "Contact with the program is ³ _____."

Navigator: "That does not authorize contacting your ⁴ _____."

11

ANSWERS / EXPLANATIONS: PAGE 103

1. submitted | 2. unconfirmed | 3. authorized | 4. employer

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Explaining a new cancer diagnosis

A Briefing checks / page 4

1. What is established?

D. A malignant tumor on pathology

The pathology confirms malignancy; staging and individual outcomes remain unresolved.

2. What does Maya request?

A. The main finding followed by a pause

Maya's stated preference is a clear main finding and time to respond.

3. What family decision is unfinished?

C. Whether a relative should join

Maya has not yet chosen whether to include a relative.

D Language checks / page 9

1. Which opening is clearest?

C. The tissue examination confirms a malignant tumor.

Naming the confirmed tumor states the established finding without obscuring it or inventing later conclusions.

2. Which response respects Maya's preference?

C. Pause after the main finding.

Maya explicitly requests time to respond before receiving more detail.

3. Which conclusion exceeds the facts?

B. The diagnosis alone establishes Maya's individual prognosis.

A confirmed diagnosis does not supply the missing stage or an individual outlook.

4. What should precede family involvement?

D. Clarifying Maya's wishes and the appropriate permissions

Family involvement must reflect the patient's wishes and the applicable information-sharing rules.

01 Conversation answers

Explaining a new cancer diagnosis

E Dialogue gaps

Pages 10-11

1. information preference. Turn 2. Information preference describes Maya's requested amount and sequence of detail for this difficult conversation.

2. malignant tumor. Turn 3. A malignant tumor is the cancerous finding established by the pathology in this fictional case.

3. biopsy. Turn 4. A biopsy provides tissue or cells for examination; here its report has established a malignant finding.

4. pathology report. Turn 6. The pathology report contains the interpretation of the examined tissue and supports the established diagnosis.

5. staging workup. Turn 8. The staging workup establishes the extent of disease and is still incomplete in this case.

6. prognostic uncertainty. Turn 10. Prognostic uncertainty means the individual future course cannot be established from the available finding alone.

7. support person. Turn 12. A support person is someone Maya chooses to involve; a relative is not automatically entitled to the information.

8. care navigator. Turn 15. A care navigator helps coordinate access and appointments without replacing the clinician's diagnostic or treatment responsibilities.

9. information overload. Turn 17. Information overload occurs when the volume of detail exceeds what a person can usefully process at the time.

10. second opinion. Turn 19. A second opinion is another qualified assessment and can be discussed without treating the request as disloyalty.

F Transfer conversation

Page 12

1. Ren

Alex: "I would like Ren to join."

Ren is the chosen partner for the next conversation in this scenario.

2. relatives

Clinician: "The permission does not automatically include other relatives."

The specified preference does not authorize broader family disclosure by itself.

3. outstanding

Alex: "Some findings remain outstanding."

The next meeting is intended to review findings that are still outstanding.

4. supplied

Clinician: "No outcome has been supplied."

The case provides an arrangement for discussion, not a new clinical result.

02 Briefing and language checks

Separating stage, grade, and prognosis

A Briefing checks / page 13

1. Which item is available?

D. A pathology report containing grade

The pathology grade is available while the staging workup is unfinished.

2. What is pending?

D. Imaging needed for staging

The brief explicitly identifies staging imaging as still pending.

3. What has Theo confused?

B. Grade with stage

Theo has interpreted the grade as though it were the completed stage.

D Language checks / page 18

1. Which contrast is correct?

C. Grade concerns microscopic features; stage concerns extent.

The two classifications address different properties and cannot replace each other.

2. What does pending imaging mean?

A. The required result is not yet available.

Pending describes availability and does not establish the result's clinical meaning.

3. Which wording preserves an indeterminate finding?

B. Its significance is not yet established.

Indeterminate means uncertainty remains and should not be converted into certainty.

4. Which claim is unsupported?

B. The grade supplies Theo's exact survival time.

Neither the grade nor the supplied facts establish an individual survival prediction.

02 Conversation answers

Separating stage, grade, and prognosis

E Dialogue gaps

Pages 19-20

- 1. Tumor grade.** Turn 2. Tumor grade describes microscopic features under the relevant system and is distinct from the cancer's extent.
- 2. cancer stage.** Turn 4. Cancer stage describes extent under the applicable system and remains incomplete while necessary evidence is pending.
- 3. Differentiation.** Turn 6. Differentiation concerns how closely tumor cells resemble normal cells and can contribute to grading in the relevant system.
- 4. local extent.** Turn 8. Local extent describes disease in the original area and is one component rather than the entirety of staging.
- 5. regional lymph node.** Turn 10. A regional lymph node belongs to the relevant nearby drainage region and differs from a distant disease site.
- 6. indeterminate finding.** Turn 12. An indeterminate finding has unresolved nature or significance and is not the same as confirmed cancer spread.
- 7. Distant metastasis.** Turn 14. Distant metastasis means established cancer spread to a distant site, which an uncertain spot does not prove.
- 8. TNM classification.** Turn 16. TNM classifies tumor, regional nodes, and metastasis in cancer-specific ways and is not a universal staging shortcut.
- 9. prognostic factor.** Turn 18. A prognostic factor is associated with the expected course but does not determine one person's exact future.
- 10. staging completion.** Turn 20. Staging completion means the necessary information has been assembled and interpreted rather than assumed from grade alone.

F Transfer conversation

Page 21

1. indeterminate

Patient: "The report calls the finding indeterminate."

The supplied report explicitly preserves uncertainty about the finding's nature.

2. reviewing

Clinician: "The team is still reviewing it."

Review is the current action, not a completed metastatic diagnosis.

3. established

Patient: "Spread has not been established."

The case provides no conclusion that the finding represents cancer spread.

4. confirmed

Clinician: "No stage change is confirmed."

A stage change cannot be inferred from the uncertain finding alone.

03 Briefing and language checks

Discussing biomarker results

A Briefing checks / page 22

1. What does the report identify?

D. A potentially relevant tumor alteration

The molecular finding may be relevant, but further interpretation is still required.

2. What is not established?

A. Suitability or availability of a particular treatment

The case leaves appropriateness and access unresolved rather than promising a treatment.

3. What kind of testing is described?

A. Tumor testing

The report concerns the tumor and does not establish an inherited finding.

D Language checks / page 27

1. Which statement preserves the result accurately?

A. The alteration may be relevant; suitability still needs review.

May be relevant preserves the finding without treating unresolved suitability as an established treatment choice.

2. Which distinction matters for relatives?

A. Tumor findings are not automatically inherited findings.

The type of testing and further assessment determine whether inherited implications are established.

3. What does uncertain significance mean?

C. Clinical meaning is not established.

The phrase preserves uncertainty rather than supporting a specific action by itself.

4. Which additional issue belongs in the discussion?

B. Evidence, clinical context, and treatment access

Those unresolved factors affect whether a possible match becomes a useful clinical option.

03 Conversation answers

Discussing biomarker results

03

E Dialogue gaps

Pages 28-29

- 1. molecular alteration.** Turn 2. A molecular alteration is a change in a molecular feature and does not alone establish an effective treatment.
- 2. actionability.** Turn 4. Actionability concerns whether a finding can usefully inform a clinical action in the actual context.
- 3. evidence level.** Turn 6. Evidence level describes the strength of support for a proposed clinical use, which can vary between findings.
- 4. targeted therapy.** Turn 8. Targeted therapy acts on a relevant molecular target but is not a guarantee of effectiveness for every patient.
- 5. treatment access.** Turn 10. Treatment access concerns whether an appropriate option can actually be obtained, separate from a report's potential match.
- 6. somatic variant.** Turn 12. A somatic variant is acquired in body cells and is distinct from a change established as inherited.
- 7. germline variant.** Turn 14. A germline variant concerns inherited genetic material, which has not been established by this tumor report alone.
- 8. variant of uncertain significance.** Turn 16. A variant of uncertain significance has an unresolved clinical meaning and should not be presented as a proven treatment target.
- 9. testing limitation.** Turn 18. A testing limitation restricts what the analysis can detect or establish and belongs in a balanced interpretation.
- 10. genetic counseling.** Turn 20. Genetic counseling provides qualified support for understanding genetic findings and implications without assuming inherited risk has already been proved.

F Transfer conversation

Page 30

1. insufficient

Patient: "The sample was insufficient for that analysis."

The report describes a sample limitation, not an absent molecular target.

2. unavailable

Clinician: "The target result is unavailable."

The analysis did not produce an interpretable result for that target.

3. negative

Patient: "That is not the same as a negative result."

Failure to obtain a result cannot be converted into a negative finding.

4. review

Clinician: "Further testing needs clinical review."

The next testing decision requires assessment rather than automatic repetition.

04 Briefing and language checks

Making treatment intent explicit

04

A Briefing checks / page 31

1. What is the proposed aim?

C. Disease control

The case names disease control and explicitly excludes a promised cure.

2. What matters particularly to Jordan?

B. Caring for a partner

Jordan's caregiving role is the stated personal priority to explore.

3. What remains undecided?

A. The final treatment choice

The discussion is ongoing and no final treatment decision is supplied.

D Language checks / page 36

1. Which statement accurately names the goal?

D. This option aims at disease control.

Disease control is the stated aim; no response or cure is promised.

2. Which question elicits a relevant priority?

A. How would the treatment demands affect caring for your partner?

Asking about the effect on caregiving links treatment demands with Jordan's stated priority.

3. Which account of consent is incomplete?

D. A signature alone proves understanding.

Signing a form cannot replace adequate explanation and attention to understanding and choice.

4. Which distinction is correct?

A. Curative intent is an aim, not a guarantee.

Intent describes the goal pursued and does not establish that it will be achieved.

04 Conversation answers

Making treatment intent explicit

E Dialogue gaps

Pages 37-38

- 1. treatment intent.** Turn 2. Treatment intent identifies the goal pursued by an option rather than an outcome that has already occurred.
- 2. Disease control.** Turn 4. Disease control means limiting growth or spread and does not mean a guaranteed permanent elimination of cancer.
- 3. caregiving responsibility.** Turn 6. Caregiving responsibility is a real practical priority that should inform the discussion of treatment demands.
- 4. Treatment burden.** Turn 8. Treatment burden includes physical, practical, and emotional demands and is relevant to an informed treatment choice.
- 5. treatment regimen.** Turn 10. A treatment regimen is the planned combination and schedule, which must be explained for the actual option rather than invented here.
- 6. treatment cycle.** Turn 12. A treatment cycle is a repeating unit of a planned schedule, not a guarantee of response or a universal length.
- 7. alternative option.** Turn 14. An alternative option is another reasonable approach that belongs in the individualized discussion before a voluntary decision.
- 8. quality of life.** Turn 16. Quality of life concerns experienced well-being and functioning, not merely a radiological measure.
- 9. Informed consent.** Turn 18. Informed consent requires a voluntary decision after appropriate explanation and questions, not merely a convenient appointment or signature.
- 10. benefit-risk balance.** Turn 20. The benefit-risk balance weighs potential benefit against possible harm without treating either as certain.

F Transfer conversation

Page 39

1. relief

Clinician: "The stated aim is symptom relief."

Symptom relief is the goal supplied for this fictional option.

2. agreed

Reese: "The schedule is not yet agreed."

The case explicitly leaves the treatment schedule unsettled.

3. demands

Clinician: "We need to explain the practical demands."

Reese requests information about the option's practical effect before deciding.

4. open

Reese: "My decision remains open."

No final choice is supplied in this ongoing discussion.

05 Briefing and language checks

Escalating a treatment-related symptom report

A Briefing checks / page 40

1. What is reported?

C. Fever and shaking after recent treatment

Lee reports symptoms; the cause has not been established.

2. What action is already taken?

B. The urgent assessment pathway is activated

The brief says the service's urgent pathway has already been activated.

3. What remains unknown?

C. Cause and laboratory results

The handoff must preserve these unknowns rather than manufacture findings.

D Language checks / page 45

1. Which opening best supports the handoff?

D. Urgent symptom report after recent treatment; the pathway is activated.

Naming the urgent report and activated pathway communicates the concern and action without inventing a diagnosis.

2. Which diagnosis cannot be asserted from this brief?

A. Confirmed neutropenia

A low neutrophil count has not been established because laboratory results are unknown.

3. What must the handoff confirm?

D. The receiving clinician and responsibility

A handoff requires an accepted next step rather than relying on a message's existence.

4. Which statement preserves causality correctly?

D. Symptoms occurred after treatment; the cause is not established.

Temporal sequence does not prove causality and does not remove the need for urgent assessment.

05 Conversation answers

Escalating a treatment-related symptom report

E Dialogue gaps

Pages 46-47

1. urgent assessment. Turn 2. Urgent assessment is the activated clinical pathway in this case, not a routine message to be reviewed later.

2. symptom onset time. Turn 4. Symptom onset time identifies when the symptoms began and must be confirmed rather than guessed during the handoff.

3. last treatment date. Turn 6. The last treatment date provides relevant clinical context and should be verified from the appropriate source.

4. causality assessment. Turn 8. Causality assessment determines whether treatment caused an event; that conclusion is not supplied by timing alone.

5. neutrophil count. Turn 10. A neutrophil count is a laboratory measurement that has not yet been supplied in this scenario.

6. infection concern. Turn 12. An infection concern is a clinical suspicion requiring assessment, not a confirmed infection diagnosis.

7. receiving clinician. Turn 14. The receiving clinician is the professional accepting the next clinical assessment and responsibility in the handoff.

8. read-back. Turn 16. Read-back repeats important details to confirm accuracy rather than assuming the message was understood correctly.

9. escalation acknowledgment. Turn 18. Escalation acknowledgment records that the urgent request was received and accepted, beyond merely attempting contact.

10. adverse event. Turn 20. An adverse event is an unfavorable occurrence during care and does not by itself prove treatment causation.

F Transfer conversation

Page 48

1. active

Nurse: "The urgent pathway is active."

The scenario begins with the actual urgent response already in progress.

2. accept

Dr Lane: "I accept the handoff."

Dr Lane explicitly accepts responsibility for receiving the clinical handoff.

3. pending

Nurse: "The laboratory results remain pending."

The results have not yet become available in the supplied facts.

4. established

Dr Lane: "The cause is not established."

Neither the symptom report nor the handoff establishes a cause by itself.

05

06 Briefing and language checks

Discussing a response-assessment scan

A Briefing checks / page 49

1. What does the report describe?

C. A measured lesion is smaller

Only the specific size comparison is supplied, not a complete response conclusion.

2. What is still under review?

D. The complete response assessment

The complete assessment has not yet been finalized by oncology.

3. What has not been agreed?

C. A treatment change

The brief supplies no agreed change to the treatment plan.

D Language checks / page 54

1. Which update is supported?

A. One measured lesion is smaller; the full assessment remains under review.

The supported statement keeps the favorable measurement separate from the unfinished overall interpretation.

2. Which claim confuses observation and classification?

D. Smaller automatically means formal partial response.

Formal response categories require their applicable criteria and the full relevant assessment.

3. Which treatment statement is accurate?

A. No change has been agreed.

The case explicitly leaves any treatment change undecided.

4. Why avoid equating complete response with cure?

B. They describe different claims about disease and future outcome.

A response category does not automatically establish a permanent individual outcome.

06 Conversation answers

Discussing a response-assessment scan

E Dialogue gaps

Pages 55-56

1. comparison scan. Turn 2. A comparison scan provides the earlier imaging reference for the reported change in the measured lesion.

2. response assessment. Turn 4. Response assessment considers the relevant evidence and criteria rather than assigning a category from one descriptive word.

3. partial response. Turn 6. A partial response must meet the applicable assessment criteria and cannot be inferred from the word smaller alone.

4. target lesion. Turn 8. A target lesion is selected for measurement under an assessment system and does not represent every relevant disease finding.

5. Non-target lesions. Turn 10. Non-target lesions are relevant disease findings assessed outside the selected target measurements and may affect interpretation.

6. Clinical response. Turn 12. Clinical response concerns changes in symptoms, signs, or functioning and adds information distinct from imaging measurements.

7. complete response. Turn 14. Complete response refers to disappearance of detectable findings under applicable criteria and has not been established here.

8. Remission. Turn 16. Remission describes reduction or disappearance of disease signs and should not be used as an automatic guarantee of permanent cure.

9. assessment criteria. Turn 18. Assessment criteria are the defined rules used to classify changes and support interpretation rather than an automatic schedule change.

10. radiological response. Turn 20. Radiological response is change assessed on imaging and will be discussed with the full clinical picture when the review is complete.

F Transfer conversation

Page 57

1. new

Patient: "The report describes a new finding."

New describes the observation supplied, not its established significance.

2. incomplete

Clinician: "The review remains incomplete."

The oncology interpretation has not yet been completed in this scenario.

3. classified

Patient: "Progression is not yet classified."

The case does not supply a formal progression category.

4. agreed

Clinician: "No treatment change is agreed."

A new observation does not by itself establish an agreed treatment change.

07 Briefing and language checks

Explaining a clinical trial invitation

A Briefing checks / page 58

1. What is the participation status?

D. Eligibility review incomplete; no consent or enrollment

The scenario keeps eligibility, consent, and enrollment as unfinished and distinct stages.

2. How does the trial assign approaches?

B. Random assignment

The brief explicitly states random assignment without describing a guaranteed superior group.

3. What worries Val?

B. Declining might harm the care relationship

Val's concern is pressure on the relationship rather than a supplied clinical result.

D Language checks / page 63

1. Which statement separates the stages correctly?

D. Eligibility is under review; Val is not enrolled.

The case gives neither completed eligibility nor consent nor enrollment.

2. Which claim creates therapeutic misconception?

C. The research exists solely to give Val the best individual treatment.

Research evaluates a question and cannot be presented as a guarantee of individualized benefit.

3. What does randomization mean?

B. Chance-based assignment under the study process

Randomization assigns groups through chance rather than individualized selection of a supposedly superior group.

4. Which response to Val's concern is appropriate?

B. Declining research does not remove the right to appropriate care.

Voluntary participation requires avoiding coercion and clarifying continuing care.

07 Conversation answers

Explaining a clinical trial invitation

E Dialogue gaps

Pages 64-65

- 1. clinical trial.** Turn 2. A clinical trial is research evaluating an intervention, and an invitation to discuss it is not an obligation to join.
- 2. eligibility criteria.** Turn 4. Eligibility criteria are entry requirements that remain under review and do not establish enrollment by themselves.
- 3. research protocol.** Turn 6. The research protocol sets out the study's purpose and procedures and should guide an accurate explanation of participation.
- 4. randomization.** Turn 8. Randomization assigns study groups through a chance-based process rather than the clinician selecting an individually preferred arm.
- 5. investigational treatment.** Turn 10. An investigational treatment is being studied for the relevant use and is not proven superior merely because it is new.
- 6. standard of care.** Turn 12. Standard of care describes accepted clinical approaches in context and belongs in the discussion of alternatives to research participation.
- 7. research coordinator.** Turn 14. A research coordinator can clarify study arrangements and visits while clinical and consent questions remain with the appropriate professionals.
- 8. withdrawal.** Turn 16. Withdrawal is ending participation through the applicable process, with its details and continuing care discussed accurately.
- 9. consent discussion.** Turn 18. A consent discussion supports understanding and a voluntary decision rather than treating a signature as the whole process.
- 10. Enrollment.** Turn 20. Enrollment is formal entry after the required study processes and has not occurred in this scenario.

F Transfer conversation

Page 66

1. screening

Jo: "I agreed to a screening visit."

Jo agreed to an eligibility-related visit, not completed study enrollment.

2. unknown

Clinician: "Your eligibility remains unknown."

The entry requirements have not yet been established as met.

3. enrolled

Jo: "I have not enrolled in the study."

The case explicitly states that formal study entry has not occurred.

4. withdrawal

Clinician: "We will explain the withdrawal process."

Jo asks about ending participation before deciding whether to participate.

08 Briefing and language checks

Discussing palliative care and priorities

A Briefing checks / page 67

1. Why is the consultation offered?

B. Symptoms and distress

The referral addresses symptoms and distress while the treatment discussion continues.

2. What does Rowan fear?

B. That treatment is being stopped

Rowan interprets palliative care as a decision to end anticancer treatment.

3. Which preference is stated?

A. More comfortable time with family

Rowan names comfort and family time, not a completed decision about other care.

D Language checks / page 72

1. Which statement explains the referral correctly?

A. Palliative care can support symptoms alongside treatment.

Palliative support can accompany other treatment and does not settle separate care decisions.

2. Which question explores the misunderstanding?

D. What did the phrase suggest to you?

Asking what the phrase suggested invites Rowan's interpretation without blame or a leading conclusion.

3. What priority should remain visible?

A. Comfortable time with family

Comfortable family time is Rowan's explicit priority in this fictional conversation.

4. Which assumption is unsafe?

D. Every relative present has automatic decision-making authority.

Presence or relationship alone does not establish authority under the relevant legal arrangements.

08 Conversation answers

Discussing palliative care and priorities

E Dialogue gaps

Pages 73-74

1. palliative care. Turn 2. Palliative care addresses symptoms and quality of life in serious illness and can accompany anticancer treatment.

2. concurrent care. Turn 4. Concurrent care means approaches can be provided alongside one another, rather than assuming one automatically replaces another.

3. symptom management. Turn 6. Symptom management addresses distressing symptoms and is one reason for this consultation while other treatment discussions continue.

4. symptom burden. Turn 8. Symptom burden is the combined effect of symptoms on daily life and helps make Rowan's stated priority concrete.

5. Hospice care. Turn 10. Hospice care has a distinct comfort-focused purpose and applicable service criteria; it is not automatically established by this referral.

6. resuscitation status. Turn 12. Resuscitation status is a separate documented care decision and is not determined by accepting a palliative-care consultation.

7. surrogate decision-maker. Turn 14. A surrogate decision-maker acts under applicable authority when required, which is not established merely by being a helpful relative.

8. family meeting. Turn 16. A family meeting is a planned conversation with the appropriate people, guided by the patient's wishes and information-sharing arrangements.

9. Caregiver strain. Turn 18. Caregiver strain describes the burden on someone providing care and can be addressed without replacing the patient's own voice.

10. goals-of-care discussion. Turn 20. A goals-of-care discussion explores priorities and appropriate care rather than automatically ending treatment or assigning a resuscitation status.

F Transfer conversation

Page 75

1. symptoms

Taylor: "I want help with my symptoms."

Symptom support is the care need Taylor explicitly identifies.

2. review

Clinician: "Treatment options remain under review."

The options are still being reviewed and no treatment decision is supplied.

3. Jo

Taylor: "I would like Jo at the meeting."

Jo is the friend Taylor wishes to include in the discussion.

4. authority

Clinician: "Attendance does not establish decision-making authority."

Being invited to a meeting does not automatically create legal decision-making authority.

09 Conversation answers

Preparing a second-opinion record

E Extended conversation

Pages 76-77

- 1. pathology report.** Turn 4. The pathology report contains the relevant tissue-assessment findings and is more specific than an abbreviated diagnosis in an appointment letter.
- 2. imaging studies.** Turn 6. Imaging studies include the acquired examinations used for review and are distinct from the written summaries describing them.
- 3. treatment summary.** Turn 8. A treatment summary records relevant prior therapy and must be checked across sites rather than assumed complete from one institution's list.
- 4. record-transfer authorization.** Turn 12. Record-transfer authorization defines the appropriate permission for sending specified information to the receiving service through the approved process.
- 5. referral packet.** Turn 16. A referral packet is the collection of relevant documents and materials required by the receiving service and may need more than one uploaded file.
- 6. Appointment confirmation.** Turn 20. Appointment confirmation establishes an actual scheduled review and is separate from a request being sent or records being received.

F Transfer exchange

Page 78

1. missing

Patient: "The imaging studies are still missing."

The receiving service has not yet received the actual imaging studies.

2. arrived

Clinician: "The report and summary have arrived."

The scenario confirms receipt of those two documents only.

3. complete

Patient: "The referral packet is not complete."

Missing requested imaging means the packet remains incomplete.

4. confirmed

Clinician: "The appointment is not yet confirmed."

No scheduled review appointment has been confirmed in this update.

10 Conversation answers

An oncology handoff after hospital discharge

E Extended conversation

Pages 79-80

1. discharge summary. Turn 3. The discharge summary records the hospital episode and discharge instructions but must be checked against the actual current plan and medicine information.

2. medication discrepancy. Turn 6. A medication discrepancy is a conflict between medicine records or instructions that requires verification rather than a guess about which version applies.

3. treatment hold. Turn 10. A treatment hold indicates an interruption under the actual plan and does not itself specify when or whether treatment should resume.

4. result ownership. Turn 14. Result ownership identifies who must review and act on a pending result so it is not lost between inpatient and outpatient services.

5. reconciled plan. Turn 18. A reconciled plan aligns verified instructions and responsibilities after discrepancies are resolved rather than combining conflicting records without review.

6. handoff acknowledgment. Turn 20. Handoff acknowledgment confirms that the receiving team obtained the information and accepted the relevant responsibilities rather than merely receiving a file.

F Transfer exchange

Page 81

1. pending

Hospital clinician: "The result is still pending."

The result is not yet available in the supplied update.

2. named

Oncologist: "The reviewing clinician was not named."

The summary omitted who was responsible for reviewing the result.

3. ownership

Hospital clinician: "We need explicit ownership."

The teams must assign responsibility rather than assume another service will act.

4. result

Oncologist: "No conclusion can be drawn from an unavailable result."

A pending result cannot support an interpretation before it exists.

11 Conversation answers

Discussing the financial burden of treatment

E Extended conversation

Pages 82-83

1. cost-related barrier. Turn 4. A cost-related barrier is a practical difficulty affecting access and is not equivalent to an informed refusal of treatment.

2. out-of-pocket costs. Turn 6. Out-of-pocket costs are amounts the patient may need to pay under the actual coverage arrangements and cannot be guaranteed from an unverified estimate.

3. patient navigation. Turn 8. Patient navigation helps coordinate practical access and support across services without replacing the clinical plan or guaranteeing eligibility.

4. Eligibility review. Turn 12. Eligibility review checks whether the program's actual criteria are met and is separate from submitting an application or receiving approval.

5. treatment tradeoff. Turn 16. A treatment tradeoff concerns the benefits, harms, and practical implications of actual options and requires an appropriate clinical discussion rather than an automatic cost-based substitution.

6. Financial toxicity. Turn 20. Financial toxicity describes the harmful financial burden associated with care and deserves attention without blaming the patient or promising unverified assistance.

F Transfer exchange

Page 84

1. submitted

Patient: "The application has been submitted."

The support team has submitted the application but has no decision yet.

2. unconfirmed

Navigator: "Eligibility remains unconfirmed."

The program has not yet established eligibility or approved coverage.

3. authorized

Patient: "Contact with the program is authorized."

The patient has permitted contact with the assistance program.

4. employer

Navigator: "That does not authorize contacting your employer."

The supplied permission does not extend to the patient's employer.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Explaining a new cancer diagnosis

Prepare for the finding: I am afraid the result is not what we hoped for.

Acknowledge emotion: I can see that this is a great deal to take in.

Offer continuity: We will arrange another conversation when the remaining results are available.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Separating stage, grade, and prognosis

Introduce the contrast: These two terms answer different questions.

Name the missing evidence: The required imaging is still pending.

Check the contrast: Which term describes extent, and which describes the tissue features?

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Discussing biomarker results

Acknowledge hope: I understand why a possible match feels important.

Limit a promise: This does not guarantee that a treatment will work.

Protect uncertainty: Uncertain significance is not the same as proven actionability.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Making treatment intent explicit

State the aim: The aim of this option is to control the disease.

Explain the burden: We should discuss visits, symptoms, and practical demands together.

Respect questions: Questions are part of the consent discussion, not an obstacle to it.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Escalating a treatment-related symptom report

Lead with urgency: I am calling about an urgent symptom report after recent treatment.

Preserve an unknown: We do not yet have the laboratory results.

Separate processes: This must not be left as a routine administrative message.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Discussing a response-assessment scan

State the observation: This measured lesion is smaller than on the comparison scan.

Avoid overstatement: Smaller does not by itself mean cured.

Check the summary: What would you tell someone about what is known and what is still pending?

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Explaining a clinical trial invitation

Introduce research: This is an option to discuss, not an obligation.

Separate stages: Eligibility review is not the same as enrollment.

Clarify withdrawal: We will explain the actual withdrawal process and remaining care arrangements.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Discussing palliative care and priorities

Ask what was heard: What did the phrase palliative care suggest to you?

Elicit a priority: What would make the coming weeks more manageable for you?

Make goals concrete: More comfortable time with family is a priority we should keep visible.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

acquired resistance	investigate acquired resistance	24
actionability	assess clinical actionability	23
adjuvant treatment	discuss adjuvant treatment	32
advance care planning	offer advance care planning	68
advance directive	review an advance directive	68
adverse event	report an adverse event	41
alternative option	compare alternative options	33
assessment criteria	apply the relevant assessment criteria	51
benefit-risk balance	explain the benefit-risk balance	33
biomarker	interpret a tumor biomarker	23
biopsy	explain the biopsy finding	5
cancer stage	establish the cancer stage	14
care navigator	contact the care navigator	6
care preference	elicit a care preference	68
caregiver strain	acknowledge caregiver strain	69
caregiving responsibility	discuss caregiving responsibilities	33
causality assessment	preserve uncertainty in causality assessment	41
clinical handoff	confirm the clinical handoff	6
clinical handoff	confirm the clinical handoff	15
clinical handoff	confirm the clinical handoff	24
clinical handoff	confirm the clinical handoff	33
clinical handoff	confirm the clinical handoff	42
clinical handoff	confirm the clinical handoff	51
clinical handoff	confirm the clinical handoff	60

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

clinical handoff	confirm the clinical handoff	69
clinical response	compare clinical response	50
clinical stage	record the clinical stage	14
clinical trial	discuss a clinical trial	59
comfort-focused care	explain comfort-focused care	68
companion diagnostic	verify a companion diagnostic	23
comparison scan	review the comparison scan	50
complete response	qualify a complete response	50
concurrent care	discuss concurrent care	68
consent discussion	allow time for the consent discussion	60
control group	explain the control group	59
curative intent	explain curative intent	32
decision capacity	assess decision capacity	33
dehydration	assess suspected dehydration	41
diagnostic certainty	distinguish diagnostic certainty	6
diagnostic disclosure	prepare for diagnostic disclosure	5
differentiation	describe cellular differentiation	14
disease control	discuss disease control	32
distant metastasis	evaluate distant metastasis	14
documented plan	review the documented plan	6
documented plan	review the documented plan	15
documented plan	review the documented plan	24
documented plan	review the documented plan	33
documented plan	review the documented plan	42

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

documented plan review the documented plan	51
documented plan review the documented plan	60
documented plan review the documented plan	69
eligibility criteria review eligibility criteria	59
emotional acknowledgment offer emotional acknowledgment	5
enrollment distinguish enrollment from screening	59
escalation acknowledgment obtain escalation acknowledgment	42
evidence level review the evidence level	23
extravasation escalate suspected extravasation	41
family meeting arrange a family meeting	69
febrile episode report a febrile episode	41
follow-up consultation arrange a follow-up consultation	6
genetic counseling arrange genetic counseling	24
germline variant assess a germline variant	23
goals-of-care discussion open a goals-of-care discussion	68
histology review the histology	5
hospice care distinguish hospice care	68
imaging finding review an imaging finding	14
immunosuppression consider immunosuppression	41
indeterminate finding explain an indeterminate finding	15
infection concern escalate an infection concern	41
information overload avoid information overload	6
information preference check the information preference	5
informed consent support informed consent	33

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

infusion reaction report a suspected infusion reaction	41
interdisciplinary support coordinate interdisciplinary support	69
investigational treatment discuss an investigational treatment	59
last treatment date verify the last treatment date	42
local extent assess local extent	14
local treatment discuss local treatment	32
localized disease describe localized disease	15
malignancy explain a confirmed malignancy	5
measurable lesion identify a measurable lesion	50
measurement variability account for measurement variability	51
metastatic disease discuss metastatic disease	15
molecular alteration characterize a molecular alteration	23
mucositis describe mucositis symptoms	41
multidisciplinary team consult the multidisciplinary team	5
neoadjuvant treatment explain neoadjuvant treatment	32
neutropenia assess for neutropenia	41
neutrophil count review the neutrophil count	41
new lesion investigate a new lesion	50
nodal involvement assess nodal involvement	15
non-target lesion assess non-target lesions	50
overall survival interpret overall survival	59
palliative care introduce palliative care	68
partial response explain a partial response	50
pathological stage distinguish pathological stage	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

pathology report review the pathology report	5
patient preference elicit the patient preference	6
patient preference elicit the patient preference	15
patient preference elicit the patient preference	24
patient preference elicit the patient preference	33
patient preference elicit the patient preference	42
patient preference elicit the patient preference	51
patient preference elicit the patient preference	60
patient preference elicit the patient preference	69
performance status assess performance status	32
predictive biomarker interpret a predictive biomarker	24
primary endpoint identify the primary endpoint	59
primary tumor identify the primary tumor	5
prognostic factor interpret a prognostic factor	15
prognostic uncertainty explain prognostic uncertainty	5
progression-free survival explain progression-free survival	60
progressive disease evaluate progressive disease	50
qualified interpreter request a qualified interpreter	6
qualified interpreter request a qualified interpreter	15
qualified interpreter request a qualified interpreter	24
qualified interpreter request a qualified interpreter	33
qualified interpreter request a qualified interpreter	42
qualified interpreter request a qualified interpreter	51
qualified interpreter request a qualified interpreter	60

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

qualified interpreter	request a qualified interpreter	69
quality of life	discuss quality of life	32
radiological response	describe radiological response	50
randomization	explain randomization	59
read-back	use read-back for key details	42
receiving clinician	identify the receiving clinician	42
recurrence	discuss recurrence	51
regional lymph node	assess regional lymph nodes	14
remission	explain remission	51
research coordinator	contact the research coordinator	60
research protocol	explain the research protocol	59
response assessment	complete a response assessment	50
restaging	explain restaging	51
resuscitation status	clarify resuscitation status	68
scan interval	confirm the scan interval	51
screening visit	arrange a screening visit	59
second opinion	arrange a second opinion	6
sequencing panel	interpret a sequencing panel	23
serious-illness conversation	structure a serious-illness conversation	68
somatic variant	explain a somatic variant	23
source attribution	preserve source attribution	6
source attribution	preserve source attribution	15
source attribution	preserve source attribution	24
source attribution	preserve source attribution	33

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

source attribution preserve source attribution	42
source attribution preserve source attribution	51
source attribution preserve source attribution	60
source attribution preserve source attribution	69
specimen adequacy confirm specimen adequacy	23
spiritual concern invite a spiritual concern	69
stable disease discuss stable disease	50
stage group confirm the stage group	14
staging completion confirm staging completion	15
staging investigation arrange a staging investigation	14
staging workup complete the staging workup	5
standard of care compare with standard of care	59
study arm describe a study arm	59
support person invite a chosen support person	6
supportive treatment coordinate supportive treatment	33
surrogate decision-maker clarify the surrogate decision-maker	68
symptom burden assess symptom burden	69
symptom management coordinate symptom management	68
symptom onset time confirm symptom onset time	42
systemic therapy explain systemic therapy	32
target lesion measure a target lesion	50
targeted therapy evaluate a targeted therapy	23
teach-back use teach-back without blame	6
teach-back use teach-back without blame	15

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

teach-back use teach-back without blame	24
teach-back use teach-back without blame	33
teach-back use teach-back without blame	42
teach-back use teach-back without blame	51
teach-back use teach-back without blame	60
teach-back use teach-back without blame	69
testing limitation explain a testing limitation	24
therapeutic misconception address therapeutic misconception	60
TNM classification explain the TNM classification	14
treatment access clarify treatment access	24
treatment burden assess treatment burden	32
treatment cycle explain a treatment cycle	32
treatment intent clarify treatment intent	32
treatment regimen review the treatment regimen	32
treatment-related toxicity assess suspected treatment-related toxicity	41
tumor grade explain tumor grade	14
tumor heterogeneity discuss tumor heterogeneity	24
tumor profiling review tumor profiling	23
urgent assessment arrange urgent assessment	42
values clarification support values clarification	69
variant of uncertain significance report a variant of uncertain significance	23
voluntary participation protect voluntary participation	60
warning statement use a clear warning statement	5
withdrawal explain withdrawal options	60

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] National Cancer Institute. Cancer Staging.

[Open the original source](#)

Terminology context for extent of disease and staging; no fictional case supplies a staging algorithm. Checked 10 October 2026.

[2] National Cancer Institute. Biomarker Testing for Cancer Treatment.

[Open the original source](#)

Background on biomarker findings, treatment relevance, and the limits of a possible match. Checked 10 October 2026.

[3] National Cancer Institute. Palliative Care in Cancer.

[Open the original source](#)

Context for supportive care alongside treatment and the distinction from hospice care. Checked 10 October 2026.

[4] National Cancer Institute. Taking Part in Cancer Treatment Research Studies.

[Open the original source](#)

Background for voluntary research participation and informed-consent discussions. Checked 10 October 2026.

Phil Stilwell

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